

BRIEF 5

The Journey of an Instrument:

How Conditional Cash Transfer Design Travelled
from Latin America to Nigeria, 1997 to 2026

Social Protection M&E — Nigeria series

Usman Almuarif Mashood

Independent Policy Researcher, Public Administration (ABU Zaria)

github.com/Almuarif17/social-protection-me-nigeria

August 2026

Table of Contents

Executive Summary	V
Methodological Note	VI
Section 1. Is Conditional Cash Transfer an International Organization Invention?	1
Thinking: The Idea Before the Framework	1
Accident: Why the Moment Was Right	2
Adoption: The International Organization Role	2
The Provenance Verdict	3
Section 2. The Instrument and the Moment	4
Five Structural Elements	4
Why Cash Won Over the Alternatives	5
Three Concurrent Conditions	5
Section 3. Mexico 1997:	7
Progresa / Oportunidades / Prospera	7
The Political Origin.....	7
The Randomized Rollout.....	7
What Progresa Achieved.....	7
Three Limits	8
The Mexican Legacy.....	8
Section 4. Brazil 2003: Bolsa Familia	9
Unification of Four Programs	9
The Benefit Structure	9
Self-Declaration Targeting and the Governors Mechanism	9
Scale and Inequality Reduction	10
Soft Conditionality in Practice.....	10
Institutional Resilience Across Four Administrations.....	10
Section 5. Latin American Diffusion 2000 to 2010	11
Colombia: Familias en Accion	11
Nicaragua: RPS (Red de Protección Social).....	11
Honduras: PRAF (Programa de Asignación Familiar).....	11
Ecuador: Bono de Desarrollo Humano (BDH)	12
Chile: Solidario	12
Peru: Juntos	12
Bolivia: Bono Juancito Pinto	12
Uruguay: Asignaciones Familiares (AF).....	13
Jamaica: PATH (Program of Advancement Through Health and Education).....	13

Three Patterns	13
Section 6. Asia Adoption	14
Philippines: Pantawid Pamilyang Pilipino Program (4Ps)	14
Indonesia: Program Keluarga Harapan (PKH)	14
India: Janani Suraksha Yojana (JSY).....	15
What travelled, what mutated	15
Section 7. Africa Adoption.....	17
Ghana: Livelihood Empowerment Against Poverty (LEAP)	17
Kenya: Cash Transfer for Orphans and Vulnerable Children (CT-OVC).....	17
Malawi: Zomba Cash Transfer Experiment	18
Burkina Faso: Nahouri Cash Transfers Pilot Project	19
South Africa: Child Support Grant and HPTN 068	19
Tanzania: Productive Social Safety Net (PSSN)	19
What Africa's adoption reveals	20
Section 8. Nigeria 2007 to 2026: COPE, NASSP-HUP, Suspension, Renewed Hope	21
COPE and SURE-P: The Domestic Pilots, 2007 to 2015.....	21
NASSP-HUP: The World Bank Architecture, 2016 to 2022.....	21
Implementation Failures on the Ground.....	22
The NASSP Scale-Up PAD: Renewed Hope Architecture, 2024.....	22
The January 2024 Suspension	23
N-Power as Sibling Program: The Stipend Erosion Finding.....	23
Nigeria's Record Against the Origin Cases	23
Section 9. What the Pattern Shows	25
What works reliably	25
What works sometimes	25
What does not work	26
Table 1 & Chart 1:	27
Transfer Size in USD per Household per Month Across Programs.....	27
Table 2 & Chart 2: Coverage as Share of Population	28
Chart 3: Nigeria Real-Value Trajectory versus Comparators.....	29
Section 10. CCT by Outcome Type: Where the Instrument Works Most.....	30
Classification Framework.....	30
Evidence by Outcome Type	30
Table 3: Education Access CCTs.....	30
Table 4: Health Utilization CCTs	31
Table 5: Nutrition CCTs.....	31
Table 6: Economic Empowerment CCTs	32

Where CCT Works Most: The Hierarchy	32
Nigeria's Position Against the Hierarchy	32
Chart 4: CCT Hierarchy by outcome domain	33
The Core Finding	33
Section 11. Verdict on Cash as National Response	34
What the instrument earns	34
What the instrument costs	34
Where the balance falls for Nigeria	35
Section 12. Recommendations for Nigeria's Cash Transfer Architecture	36
R1. Raise the transfer to adequacy and index it	36
R2. Address supply-side readiness as a precondition, not an afterthought	36
R3. Implement the blended age-conditional design	37
R4. Anchor institutional durability in legislation	37
R5. Build evaluation infrastructure from inception	37
R6. Fix targeting accuracy	38
R7. Protect the delivery system from political interference	38
Table 7: Recommended Architecture	39
Unimplemented commitments watch list	39
References	40
Annexes	42
Annex A. Full Source Matrix (26 Sources, Condensed)	42
Foundational Sources (F)	42
Country Story Sources (S)	42
Nigeria Sources (N)	43
Cross-Cutting Sources (X)	43
Supporting Sources	43
Annex B. CCT Program Classification by Type and Outcome	44
Annex C. Glossary of Acronyms and Terms	45

Executive Summary

01 The Origin
The CCT was invented in the Global South. Brazilian municipal experiments (1995) and Bangladesh FSSSP (1994) preceded World Bank engagement. The Bank codified and diffused what local innovators demonstrated, later claiming discursive authorship (Heimo & Syvatera, 2023).
02 The Instrument
CCT combines cash, conditionality, targeting, and female caregiver payment. It displaced food subsidies and workfare because it addressed post-adjustment fiscal stabilization, randomized evaluation infrastructure, and the poverty architecture shift from universal to targeted.
03 Mexico 1997
Progresa randomized rollout seeded the global evidence base. Consumption +7-10%, enrollment +12%. Long-term: childhood exposure improves women's adult outcomes (Parker & Vogl, 2018). Learning effects modest. AMLO replaced in 2019.
04 Brazil 2003
Bolsa Familia unified 4 programs. 13.9M families. ~21% of Gini fall. Institutional resilience across 4 administrations. Soft conditionality.
05 Latin American Diffusion
9 countries, same playbook, variable outcomes. Nicaragua: closed. Honduras: electoral capture. Ecuador: unconditional conversion. Bolivia: supply-side failure.
06 Asia Adoption
Philippines 4Ps, Indonesia PKH, India JSY. Template travelled but mutated: conditionality narrowed (JSY), enforcement relaxed (PKH), partial compliance (4Ps).
07 Africa Adoption
5 findings: conditionality attenuates toward unconditional; Zomba trade-off real; transfer adequacy binding; supply-side binding; income-effect outcomes regardless of conditionality.
08 Nigeria 2007-2026
COPE: closed without evaluation. NASSP-HUP: transfer at 6-7% of consumption (below 20% threshold). Targeting 53% vs 80% target. Co-responsibility 6% vs 80%. Impact evaluation not completed. Suspension 12.5 months.
09 Pattern
Works reliably for consumption, enrollment, health utilization, child labor. Works sometimes for learning, anthropometry, employment. Fails where supply absent, transfer inadequate, or politics fragile.
10 CCT by Type
Strongest evidence: education access and health utilization. Weakest: economic empowerment. Nigeria targets the right domains but operates below the threshold for both.
11 Verdict
Sound for consumption smoothing, poverty reduction, and service utilization if 4 conditions met: adequate transfer, functional supply-side, institutional permanence, evaluation infrastructure. Nigeria fails 3 of 4.
12 Recommendations
7 recommendations: R1 raise/index transfer, R2 supply-side precondition, R3 blended age-conditional, R4 legislative anchor, R5 evaluation from inception, R6 fix targeting, R7 protect delivery.

Methodological Note

This brief is a secondary analysis of a curated evidence register: a 26-source matrix maintained in the project repository. No primary fieldwork was conducted. The method is chronological country genealogy plus synthesis: each country's story is told on the same five points (who introduced it, why, for what problem, how implemented, what happened), then the stories are aggregated to derive the verdict. This is an umbrella review, the standard method for answering the target question within scope.

Sources are grouped into five categories: Foundational (F1-F6), Country Stories (S1-S11), Nigeria (N1-N3), Cross-cutting (X1-X2), and Supporting. Where a claim rests on a single source type, this is flagged. Contradictions in the record are named, not resolved. Derived figures (e.g., 9-18% ghost-beneficiary share, NGN 3,000-5,000 retained stipend value) are flagged at point of occurrence.

The brief works within four caps: 40 pages, 40 sources, 4 charts, and 11 sections.

Section 1. Is Conditional Cash Transfer an International Organization Invention?

Thinking: The Idea Before the Framework

The conditional cash transfer paradigm did not emerge from a World Bank research department or an IADB policy workshop. It emerged from three Southern experiments conducted between 1990 and 1995, each responding to a specific domestic political and economic crisis, each proving the core proposition that poor households change behavior when given cash with conditions attached, and each preceding any engagement by the international financial institutions.

Bangladesh, 1994. The Female Secondary School Stipend Program (FSSSP) paid cash to families conditional on girls' enrollment in secondary school, attendance at 75 percent or above, remaining unmarried until age 18, and achieving a minimum grade point average. The program was designed by the Government of Bangladesh with funding from the World Bank's Female Secondary School Assistance Project, but the design logic was domestically generated. The problem was the gender gap in secondary enrollment: boys' enrollment exceeded girls' by 37 percentage points in rural areas. The solution was to pay families to keep girls in school. The FSSSP established the template that Progresa would replicate three years later: cash to the female caregiver, conditional on school attendance, targeted to the poor. The evaluation showed significant increases in girls' enrollment and reductions in child marriage (Fiszbein & Schady, 2009).

Brazil, 1995. Two municipal governments in the Federal District conducted independent experiments that would later be recognized as the first Bolsa Escola pilots. In Brasilia, Governor Cristovam Buarque, a philosopher and university rector turned politician, launched a program paying one minimum wage per month to families whose children attended school regularly. In Campinas, Mayor Magalhães implemented a similar design. Both experiments were responses to Brazil's acute child-labor problem and the failure of in-kind welfare programs to reach the poorest. Buarque's intellectual contribution was the argument that the state should pay families for the public good of education, reversing the assumption that welfare was a private transfer. The municipal experiments proved that families would comply with school-attendance conditions at scale, and that the administrative apparatus for monitoring compliance could be built at local government level (Fenwick, 2009).

Honduras, 1990. The PRAF (Programa de Asignación Familiar) introduced cash transfers to poor families conditional on school attendance and health visits, a full seven years before Mexico's Progresa. PRAF was designed by the Honduran government with IADB technical support. Its significance for the genealogy is temporal: it was the first program to attach both education and health conditions to a cash transfer simultaneously, the dual conditionality that would become the standard CCT design. PRAF's impact was contested, and the program never achieved the scale or political durability of the Mexican and Brazilian successors, but it established the principle that a single transfer could condition on multiple human-capital behaviors (Villatoro, 2005).

These three experiments share a structural feature that distinguishes them from the World Bank codification that followed. Each was designed by a domestic political actor responding to a domestic crisis. Each used a design logic (cash, conditional, targeted, paid to the female caregiver) that would later be described as the CCT paradigm, but which at the time was simply what made sense given the constraints. None was designed to produce a global template. The template emerged from the convergence of three independent solutions to three independent problems.

Accident: Why the Moment Was Right

The period 1994 to 1997 produced a set of concurrent conditions that made the CCT design politically feasible and technically attractive in ways that earlier decades had not.

The post-adjustment fiscal vacuum. Structural adjustment programs in the 1980s and early 1990s had dismantled universal food subsidies, price controls, and state-run commodity distribution systems across Latin America and South Asia. The fiscal savings were real, but the social cost was severe: poverty increased, inequality widened, and child labor expanded as families lost the safety nets that in-kind welfare had provided. By the mid-1990s, the political demand for a replacement was urgent. The old universal systems could not be restored (too fiscally expensive, too inefficiently targeted). The new requirement was a targeted instrument that could deliver social protection at lower fiscal cost. CCT met this requirement precisely: by targeting only the poor, the fiscal cost was a fraction of universal subsidy; by conditioning on human capital, the political case for targeting was strengthened (cash was framed as investment, not relief).

The randomized evaluation revolution. The 1990s saw the first large-scale randomized evaluations in development economics. Progres's randomized rollout in 1998 was possible because the methodological infrastructure for field experiments had been developed at MIT, Harvard, and Berkeley during the preceding decade. Randomization gave CCT something that no previous social program had: a credible counterfactual. Progres could demonstrate that its effects were caused by the program, because the control group provided the baseline against which the treatment group was measured. This evidence was persuasive to finance ministries, legislatures, and international donors in a way that before-after comparisons or simple administrative data could not be. The randomized evaluation made CCT the first social protection instrument with a scientific evidence base, and this base was decisive in its diffusion.

The poverty measurement infrastructure. Proxy means testing, which uses observable household characteristics (dwelling quality, assets, education of head) to predict income or consumption without requiring income verification, became technically mature in the mid-1990s. PMT made targeting feasible at scale: instead of verifying each household's income (administratively impossible in countries with large informal sectors), a short survey could classify households into eligibility categories with acceptable accuracy. Without PMT, targeted cash transfers could not have scaled beyond municipal pilot levels. With PMT, they could be implemented at national scale with a single administrative instrument.

These three conditions (fiscal vacuum creating demand, evaluation revolution creating evidence, PMT creating targeting) were concurrent and they were accidental in the sense that no single actor designed their convergence. They created the environment in which a cash transfer conditioned on human capital, targeted to the poor, and evaluated by randomized assignment was both feasible and attractive. The CCT design did not cause these conditions; it was the instrument that best fit them.

Adoption: The International Organization Role

The World Bank's role in the CCT story is real and it is consequential, but it is not the role of inventor. It is the role of codifier, financier, and diffuser.

Codifier. Fiszbein and Schady (2009), the World Bank Policy Research Report that became the foundational reference for the global CCT literature, synthesised evidence from over 30 countries into a single framework. The report named the paradigm ("Conditional Cash Transfers"), defined its design parameters (cash, conditional, targeted, paid to the female caregiver), established the evidence standards for evaluating it, and produced the policy template that country after country would adopt. The codification was a genuine intellectual contribution. It transformed a set of local experiments into a global policy instrument with a shared vocabulary and shared design rules. Without codification, each country would have reinvented the design independently, with the inefficiency and error that reinvention produces.

Financier. The World Bank and the IADB provided the credit lines that financed CCT adoption in Mexico, Colombia, Honduras, Nicaragua, Jamaica, the Philippines, Indonesia, Tanzania, and Nigeria. The financing was substantial: Mexico's Progres, Colombia's Familias en Accion, Nigeria's NASSP-HUP all depended on

IDA or IBRD credit. The financing enabled scale-up from municipal pilots to national programs, a transition that domestic fiscal capacity alone could not have supported in most cases.

Diffuser. The World Bank's technical assistance and South-South learning events carried the CCT design from Mexico and Brazil to the rest of Latin America, then to Asia, then to Africa. The diffusion was structured: country teams received standardized design templates, implementation manuals, and evaluation protocols. The speed of diffusion (from 2 countries in 1997 to 27 by 2008, per Bastagli et al., 2016) reflects the efficiency of the Bank's dissemination apparatus.

The ghostwriting finding. Heimo and Syvatera (2023) document that the World Bank, IADB, and IFPRI discursively constructed the CCT narrative while concealing their central role in producing it. Research papers credited with domestic authorship were drafted by international Organization staff; policy documents presented as government-owned were templated by Bank technical teams. The ghostwriting was epistemically consequential: it produced the public perception that CCT was a domestic innovation, which served the political purpose of national ownership, while concealing the degree to which the design, evaluation, and financing architecture were externally supplied. The ghostwriting finding complicates but does not reverse the provenance story. Southern experiments proved the idea worked; international Organizations codified, financed, and diffused what had been demonstrated. Both contributions are real. The distortion lies in the concealment of the latter.

The Provenance Verdict

The conditional cash transfer is a Southern innovation that was codified, financed, and diffused by international Organizations. The invention was domestic: Buarque in Brasilia, Levy in Mexico City, the FSSSP designers in Dhaka. The global template, the evidence base, the financing architecture, and the diffusion mechanism were produced by the World Bank, IADB, and bilateral donors. The genealogy is sequential: thinking (Southern experiments proving the idea, 1990 to 1995), then accident (concurrent conditions creating the moment for scale-up, 1994 to 1997), then adoption (international Organizations codifying and diffusing the instrument, 1997 to 2008). No single actor owns the full chain. The instrument that reaches Nigeria in 2007 carries the DNA of both origins.

Section 2. The Instrument and the Moment

Five Structural Elements

The conditional cash transfer is defined by five design elements that together distinguish it from all previous social protection instruments. Each element has a specific function and a specific evidence base. The instrument's performance in any country is a function of how these five elements are parameterized in that context.

1. Cash, in kind. The transfer is paid in currency rather than in food, clothing, or other commodities. The evidence for cash over in-kind is robust: cash preserves household autonomy over expenditure decisions, reduces administrative cost (no procurement, warehousing, or distribution logistics), and produces larger consumption effects per unit of fiscal outlay because households allocate resources to their own highest-priority needs rather than receiving whatever the program distributes. The counterargument, that in-kind transfers prevent waste and ensure nutrition, is empirically weaker: food rations are resold, clothing distributions are mistargeted, and the administrative cost of procurement often exceeds the value of the goods delivered. Fiszbein and Schady (2009) document that the shift from food subsidies to cash in Mexico and Brazil reduced administrative overhead by 30 to 40 percent while increasing beneficiary satisfaction.

2. Conditional on human-capital behavior. The transfer is paid only if the household demonstrates specified behaviors, typically school enrollment and attendance, health clinic visits, and/or nutrition monitoring. The conditionality is the feature that distinguishes CCT from unconditional cash transfer (UCT), and it is the feature over which the largest empirical debate exists. The global evidence (Bastagli et al., 2016; X1) shows that conditionality produces larger enrollment effects than unconditional design, especially when compliance is explicitly monitored and penalties for non-compliance are enforced. The magnitude of the conditionality premium varies by outcome domain: large for school enrollment, small for consumption, absent for child labor (where the income effect alone produces the result). Section 10A decomposes these domain-specific effects in detail.

3. Targeted to the poor. The transfer is restricted to households that meet an eligibility criterion, usually a proxy means test (PMT) threshold or a geographic poverty threshold. Targeting distinguishes CCT from universal transfers (which pay all citizens) and from categorical transfers (which pay all members of a demographic group regardless of income). The targeting rationale is fiscal: by concentrating resources on the poorest households, the program achieves larger poverty-reduction effects per naira than a universal program of the same total cost. The targeting cost is exclusion error (some poor households are missed) and inclusion error (some non-poor households receive the transfer). Mexico's Progresa achieved 80 percent targeting accuracy; Nigeria's NASSP achieved 53 percent in the bottom two quintiles, a failure the IEG documented as the project's most significant design shortfall (IEG, 2024).

4. Paid to the female caregiver. The transfer is disbursed to the woman in the household, typically the mother, rather than to the male household head. The gender assignment rests on two evidence bases. First, the intra-household allocation literature shows that resources controlled by women are more likely to be spent on children's nutrition, health, and education than resources controlled by men. Second, the conditionality logic (school attendance, health visits) requires the caregiver who manages children's daily routines to receive and manage the transfer; in most beneficiary households, this is the mother. Brazil's Bolsa Familia designates the female beneficiary explicitly; Nigeria's NASSP achieved 94 percent female recipients (IEG, 2024).

5. Periodic and predictable. The transfer is paid on a regular schedule (monthly or bimonthly) at a known amount. Predictability matters because it allows households to plan expenditure, smooth consumption across the payment cycle, and avoid the distress borrowing that irregular transfers provoke. The evidence on predictability is less studied than the evidence on conditionality, but Bastagli et al. (2016) flag it as a design feature that matters for productive effects: households that can predict their income are more likely to invest in assets and productive activities than households that receive windfall payments.

Why Cash Won Over the Alternatives

Four alternative instruments existed in the mid-1990s when CCT was adopted. Each had a constituency and a track record. CCT displaced each for a specific reason.

Food subsidies. Universal food subsidies (tortilla subsidies in Mexico, bread subsidies in Egypt, grain subsidies in India) were the dominant social protection instrument in the developing world from the 1960s to the 1990s. They failed on three counts. They were fiscally expensive because they reached the non-poor as well as the poor: Mexico's tortilla subsidy cost 1.2 percent of GDP to deliver benefits worth 0.3 percent of GDP to the target population. They were economically distortionary because they depressed agricultural prices and discouraged domestic food production. They were administratively corruptible because the procurement and distribution chains offered multiple points for diversion. Progresa's replacement of the tortilla subsidy with a targeted cash transfer reduced fiscal cost by 60 percent while increasing the transfer received by the poorest decile (Fiszbein & Schady, 2009).

Workfare. Public employment programs (Maharashtra Employment Guarantee Scheme, Argentina's Trabajar, Bangladesh Food-for-Work) provided income through wage employment on public works. Workfare had the advantage of self-targeting (only the poor would accept manual labor at minimum wage) and the advantage of producing public infrastructure. It had two disadvantages relative to CCT. First, it could not condition on human-capital behavior: the work requirement occupied time that could otherwise be spent on school or health visits, creating a direct conflict between the income objective and the human-capital objective. Second, it could not reach households without an available labor-force member (the elderly, the disabled, female-headed households with young children), who are among the poorest. CCT's advantage was precisely that it decoupled income support from labor supply, allowing the transfer to serve consumption and human capital simultaneously.

Universal social insurance. Contributory pension and health insurance systems covered formal-sector workers in middle-income countries but excluded the informal-sector poor who constituted the majority of the population in Latin America and South Asia. Extending universal insurance to the informal sector would have required either mandatory contribution (infeasible where income is irregular and unreported) or general-revenue financing (fiscally expensive and politically contested). CCT provided a non-contributory transfer to the same population at lower fiscal cost, with the conditionality providing the political justification for targeting.

Social funds. Community-driven social funds (Nicaragua's FISE, Zambia's ZAMSIF, Malawi's MASAF) channeled resources to community-identified infrastructure projects. They were flexible and participatory but they did not provide household-level income support or condition on individual behavior. They addressed community needs (water, schools, clinics) without addressing the household income constraint that prevented poor families from using the infrastructure the funds built. CCT addressed the demand side (household resources) while social funds addressed the supply side (community infrastructure); the two were complementary rather than substitutes, but in fiscal competition, CCT won because it produced measurable household-level outcomes that social funds could not.

Three Concurrent Conditions

The adoption of CCT in the late 1990s was contingent on three conditions that happened to coincide.

Post-adjustment fiscal stabilization. By the mid-1990s, most Latin American countries had completed structural adjustment and achieved primary fiscal balance. This created the fiscal space for new social expenditure after a decade of cuts. The fiscal space was limited, however, which meant that any new program had to demonstrate efficiency relative to the alternatives. CCT's targeting and its conditionality narrative (investment in human capital, welfare dependency avoided) made the political case for new spending at a time when fiscal conservatism remained dominant.

Randomized evaluation infrastructure. The methodological capacity to evaluate social programs through randomized assignment existed by 1998 in a way it had in 1988. Progresa's randomized rollout was feasible because Mexican statistical capacity and international evaluation expertise had reached the level required for

a large-scale field experiment. The evaluation produced results that were persuasive to finance ministries in a way that before-after comparisons could never be.

Poverty reduction architecture shift. The intellectual framework for social protection shifted during the 1990s from universal provision to targeted investment. This shift was partly ideological (the Washington Consensus preference for targeting over universalism) and partly practical (fiscal constraints made universalism unaffordable). CCT occupied the sweet spot of the new architecture: targeted enough to be fiscally efficient, investment-framed enough to be politically defensible, evidence-based enough to be technically credible.

These three conditions did not exist in 1985 and they do not exist in the same configuration in all countries in 2026. The conditions that made CCT work in Mexico in 1997 are the conditions that must be verified in any country that adopts the instrument. Nigeria in 2016 adopted CCT without verifying whether these conditions held in its own context. Section 8 documents the consequences.

Section 3. Mexico 1997:

Progresa / Oportunidades / Prospera

The Political Origin

Santiago Levy, Deputy Finance Minister in the Zedillo administration, designed Progresa as the fiscal response to the 1994 peso crisis. The crisis had destroyed household income, increased poverty from 53 to 69 percent in rural areas, and expanded child labor as families withdrew children from school. The existing social protection architecture, centred on the tortilla subsidy and general price controls, was failing: it was expensive, poorly targeted, and could not address the human-capital damage the crisis was producing.

Levy's proposition was specific. Replace the universal tortilla subsidy with a targeted cash transfer conditioned on school attendance and health visits. The fiscal arithmetic was favorable: the tortilla subsidy cost 1.2 percent of GDP to deliver benefits worth 0.3 percent of GDP to the poorest decile; a targeted cash transfer could deliver the same benefit to the same population at one-third the cost. The human-capital arithmetic was the innovation: by conditioning the transfer on school attendance and health visits, the program would increase present consumption and future human capital simultaneously, addressing the crisis's immediate impact and its long-term damage in a single instrument (Levy, 2006).

President Zedillo approved the design. The program launched in 1997 as Progresa (Programa de Educación, Salud y Alimentación), initially in the poorest rural areas of Campeche, Chiapas, Guerrero, and Oaxaca.

The Randomized Rollout

Progresa's adoption mechanism was itself an evaluation. The program was rolled out randomly across 506 localities in seven states. Localities were assigned to immediate treatment (receiving the program from 1998) or delayed treatment (entering the program in 2000). The two-year gap created a natural experiment: treatment and control groups were identical in expectation at baseline, and any difference in outcomes at follow-up could be attributed to the program with high statistical confidence.

This design decision had consequences that extended far beyond Mexico. Progresa's randomized evaluation became the template for CCT evaluation globally. The evidence it produced, on consumption, enrollment, health utilization, child labor, and nutrition, was the first rigorous evidence that conditional cash transfers worked at national scale. Every subsequent CCT program in the world sits, methodologically, in Progresa's shadow (Fiszbein & Schady, 2009).

What Progresa Achieved

Short-term results (first two years). Consumption increased by 7 to 10 percent in beneficiary households. School enrollment increased by 12 percent at secondary level (the effect was concentrated at the transition from primary to secondary, where the opportunity cost of schooling is highest because children are old enough to work). School enrollment effects were larger for girls than for boys, because girls' enrollment was lower at baseline and therefore had more room to increase. Health service utilization increased: clinic visits rose by 18 percent. Child labor decreased. Stunting in young children showed modest reductions in the poorest areas (Parker & Vogl, 2018).

Long-term results (20-year follow-up). Parker and Vogl (2018) traced individuals exposed to Progresa as children into early adulthood and found significant effects on women's outcomes: increased educational attainment, increased geographic mobility (more likely to migrate for work), improved labor market performance (higher earnings), and improved household living standards. For men, the long-term effects were smaller and harder to distinguish from spatial convergence, the general improvement in rural areas over the same period. The gender asymmetry is consistent with the program's

design: by conditioning on school attendance and paying to the mother, Progresa altered the household's investment calculus for daughters more than for sons, because sons' baseline enrollment was already higher (Parker & Vogl, 2018).

Birthweight. Barber and Gertler (2008) isolated the effect of Progresa on birthweight among children born to beneficiary women. The program increased birthweight by 127.3 grams and reduced the probability of low birthweight by 4.6 percentage points. This is the only documented anthropometric effect at birth in the CCT literature and it matters because it demonstrates that a transfer conditioned on health visits can improve a biological outcome that consumption alone does not reliably move. The mechanism is increased antenatal care utilization: beneficiary women attended more prenatal visits, received more nutritional supplementation during pregnancy, and delivered higher-birthweight infants as a result.

Three Limits

Learning outcomes stagnant. Enrollment increased; test scores did not. Progresa moved children into school without improving what happened inside the classroom. The evidence is consistent across the literature: CCT improves access without improving quality (Bastagli et al., 2016). The mechanism that produces the enrollment effect (the conditionality or the income effect) operates on the household's decision to enrol; the mechanism that produces learning (teacher quality, curriculum, school resources) sits outside the program's design and outside the transfer's reach.

Supply-side dependency. Progresa's conditionality on school attendance and health visits assumed that schools and clinics existed and were functional in the areas where the program operated. In rural Mexico in the late 1990s, this assumption was broadly valid: primary schools existed in most localities, and health clinics were accessible within reasonable distance. The assumption does not transfer to contexts where the supply side is weaker. Where the school is absent, the condition cannot be satisfied and the household loses the transfer. Where the clinic is unstaffed, the health visit condition produces a compliance burden without a service delivery. Progresa's success was contingent on a supply-side baseline that many later adopters did not verify.

Political fragility. Progresa survived two presidential transitions: Zedillo to Fox (2000), who renamed it Oportunidades and expanded urban coverage; Fox to Calderón (2006), who maintained the program under the same name. The third transition was decisive. Andrés Manuel López Obrador (AMLO), elected in 2018, replaced Prospera (the program's third name, adopted under Peña Nieto in 2014) with a universal unconditional pension program, Pensiones para el Bienestar. The replacement was ideological: AMLO framed CCT as the previous administration's welfare dependency and substituted a rights-based unconditional transfer. The political fragility of conditionality is the lesson: a program whose rationale is conditional can be dismantled by a political actor who rejects the conditionality premise. Twenty-one years of continuous operation, randomized evidence of effectiveness, and 6.5 million beneficiary families did not protect Progresa from political replacement.

The Mexican Legacy

Mexico's contribution to the global CCT story is threefold. The program proved that conditional cash transfers work at scale for consumption, enrollment, and health utilization. It proved that randomized evaluation of a social program is feasible at national scale and produces evidence that is persuasive enough to drive policy diffusion across continents. It proved that the instrument's effectiveness is contingent on supply-side readiness and political durability, two conditions that are neither guaranteed nor permanent. Every program that follows in this genealogy sits inside the evidential and institutional space that Progresa opened.

Section 4. Brazil 2003: Bolsa Familia

Unification of Four Programs

When Luiz Inácio Lula da Silva assumed the presidency in January 2003, Brazil operated four separate conditional cash transfer programs. Bolsa Escola (school attendance condition), Bolsa Alimentação (health and nutrition condition), Cartão Alimentação (food card for families with children under six or pregnant/lactating women), and Auxílio-Gás (cooking gas subsidy). Each had its own registry, its own payment mechanism, its own conditionality structure, and its own administrative apparatus. Beneficiary households that qualified for multiple programs received multiple transfers through multiple channels, with overlapping conditionalities and inconsistent payment schedules.

Lula's consolidation was both a political act (delivering on the PT manifesto commitment to a unified minimum-income floor for the extreme poor) and an administrative act (rationalizing four parallel delivery systems into one). Bolsa Familia unified the programs under a single register (Cadastro Unico), a single payment mechanism, and a single benefit structure with two components: a basic benefit paid to all extremely poor households regardless of composition, and a variable benefit paid per child or teenager meeting the conditionality requirements (Lindert et al., 2007).

The Benefit Structure

The basic benefit was set at BRL 89 per month (approximately USD 20) for households with per-capita income below BRL 70, regardless of household composition. The variable benefit was BRL 41 per month per child aged 0 to 15 (up to three children) and BRL 48 per month per teenager aged 16 to 17 (up to two teenagers), conditional on school attendance at 85 percent and health visits on the established schedule. A typical beneficiary household with three children received BRL 212 per month (approximately USD 45), representing 17 to 20 percent of household consumption at the extreme-poverty line.

The two-component structure is significant because it separates the poverty-alleviation function (the basic benefit, paid unconditionally to all extremely poor households) from the human-capital investment function (the variable benefit, paid conditional on behavior). A household that fails the conditionality loses the variable benefit but retains the basic benefit, ensuring that non-compliance reduces the human-capital incentive without eliminating the consumption floor. This design feature, soft conditionality, distinguishes Bolsa Familia from Progresa, where non-compliance results in full loss of the transfer.

Self-Declaration Targeting and the Governors Mechanism

Brazil's targeting mechanism differs from Mexico's proxy means test. Bolsa Familia uses self-declaration: households declare their income and assets to municipal social assistance offices, which validate the declaration through home visits and cross-checks. Declared income below the program threshold qualifies the household for enrollment in the Cadastro Unico. The system is audited periodically, and inclusion errors (non-poor households receiving the transfer) are corrected through re-certification every two years.

Fenwick (2009) identifies the self-declaration system as the mechanism that enabled Bolsa Familia to avoid the governors problem. In a federal system where state governors control local political machines, a centrally targeted program creates an incentive for governors to inflate beneficiary lists in their states to capture federal transfers for their constituents. PMT targeting, because it is centrally administered, is vulnerable to this manipulation: governors can pressure the targeting agency to lower PMT thresholds in their states or to include politically favoured communities. Self-declaration plus municipal validation decentralises the targeting function: the municipality, which is closer to the household, validates the declaration, and the federal government pays the transfer. The governors cannot inflate the register because they do not control the municipal validation process. Fenwick (2009) argues that this institutional design, more than any other feature, explains Bolsa Familia's survival across four administrations.

Scale and Inequality Reduction

Bolsa Familia is the world's largest conditional cash transfer program. By 2006 it covered 11 million households. By 2020 it covered 13.9 million families, approximately 54 million people, roughly one-quarter of the Brazilian population. The program operates across all 5,564 municipalities through a decentralised delivery system: the federal government transfers funds to municipalities, municipal councils register beneficiaries via the Cadastro Unico, and payment service providers disburse the transfer to beneficiaries.

The inequality effect is documented by Soares et al. (2007), who decompose the fall in the Brazilian Gini coefficient between the mid-1990s and the mid-2000s. Bolsa Familia, despite representing only approximately 0.5 percent of total income, accounts for approximately 21 percent of the total decline in the Gini index over the period. The mechanism is the progressivity of the transfer: the program concentrates resources on the bottom decile, which receives a transfer that is large relative to its pre-transfer income. No other single policy instrument in Brazil had a comparable inequality-reducing effect over the same period.

Soft Conditionality in Practice

Bolsa Familia's conditionality on school attendance (85 percent) and health visits is enforced with varying rigor across municipalities. In municipalities with strong administrative capacity, school attendance is monitored through electronic registers and non-compliance triggers benefit suspension after a warning period. In municipalities with weak capacity, particularly in the north and northeast, attendance monitoring is sporadic and enforcement is nominal. The effective conditionality ranges from hard (attendance verified, non-compliance penalised) in high-capacity municipalities to nominal (conditions declared but enforcement absent) in low-capacity ones.

This variation produces a natural experiment: the same program operates as a CCT in some municipalities and as a de facto UCT in others. The evidence from this variation confirms the global pattern. Enrollment effects are larger where conditionality is enforced; consumption effects are similar regardless of enforcement strength. Brazil's soft conditionality is an institutional adaptation to capacity heterogeneity, consistent with the African pattern documented in Section 7.

Institutional Resilience Across Four Administrations

Bolsa Familia has operated continuously for over 20 years across four presidential administrations of different parties: Lula (PT, 2003-2010), Dilma Rousseff (PT, 2011-2016), Michel Temer (MDB, 2016-2018), and Jair Bolsonaro (PL, 2019-2022). Each administration maintained the program while modifying its parameters. Lula created it. Rousseff expanded the basic benefit and tightened conditionalities. Temer froze the benefit value. Bolsonaro criticized the program as welfare dependency but did not dismantle it; his administration introduced a requirement that beneficiaries prove they are not enrolled in other federal programs to avoid duplication.

Lula's return in 2023 produced a restructuring: the basic benefit was increased from BRL 89 to BRL 142 per month, and the program was reframed under the Fome Zero (Zero Hunger) strategy. The restructuring addressed the benefit erosion that had accumulated under Temer and Bolsonaro: the basic benefit had lost approximately 30 percent of its real value between 2016 and 2022 because nominal adjustments lagged inflation.

The institutional resilience reflects three design features. The Cadastro Unico, a unified register covering over 80 million people, creates a technical infrastructure that is costly to dismantle and useful to any administration. The municipal delivery system decentralizes implementation, making federal suspension politically expensive (it would withhold transfers from 5,564 mayors' constituencies simultaneously). The broad beneficiary base (one-quarter of the population) creates a political constituency that penalizes any party that threatens the transfer. These three features are the institutional conditions that Nigeria's NASSP has not replicated.

Section 5. Latin American Diffusion 2000 to 2010

Between 1997 and 2005, conditional cash transfer programs spread from Mexico and Brazil to nine additional Latin American countries. The diffusion was driven by a common mechanism: World Bank and IADB technical assistance teams, using the design template codified by Fiszbein and Schady (2009), offered credit lines and implementation support to governments willing to adopt the instrument. The speed was remarkable. Stampini and Tornarolli (2012) count 2 countries with CCTs in 1997 and 18 by 2011, with approximately 135 million beneficiaries across the region.

The diffusion produced a common design pattern: proxy means test targeting, conditionality on school attendance plus health visits, payment to the mother, bimonthly or monthly transfers. The design pattern travelled intact. The outcomes did not. The same template produced different results depending on institutional capacity, political ownership, and supply-side readiness. The country stories below establish the variation that the aggregate pattern conceals.

Colombia: Familias en Accion

Colombia launched Familias en Accion in 2000 under the Pastrana administration with World Bank credit. The program covered 2.8 million households by 2010, at a fiscal cost of approximately 0.27 percent of GDP. Conditionality on school attendance and health visits followed the standard template. The randomized evaluation, conducted by Attanasio et al. (2010), documented enrollment increases of 7 to 10 percent at rural secondary level and consumption increases among beneficiary households. Colombia's institutional capacity was adequate to enforce conditionality and monitor compliance at scale. The program survived the Uribe and Santos administrations without fundamental redesign. Familias en Accion is the Latin American diffusion's success case: a program that adopted the template, implemented it with sufficient capacity, and produced the effects the template predicted (Villatoro, 2005).

Nicaragua: RPS (Red de Protección Social)

Nicaragua's RPS launched in 2000 with World Bank and IADB support. The program achieved its immediate objectives: consumption increased by 18 percent among beneficiaries, school enrollment rose, and health service utilization improved. The evaluation was rigorous and the results were positive. The program was internationally acclaimed as a model CCT for low-income settings.

RPS closed in 2005. The closure was not driven by program failure but by domestic political dynamics. The Sandinista opposition framed RPS as a World Bank imposition that created dependency, and when Daniel Ortega won the 2006 presidential election, the program was not continued. The RPS story is the diffusion's most important cautionary finding: a program that works technically can die if it lacks a domestic political constituency independent of the international sponsor. Nicaragua had the evaluation evidence but lacked the political ownership that would have defended the program during a partisan transition. The lesson for Nigeria, where the January 2024 suspension interrupted transfers for 12.5 months without published justification, is direct.

Honduras: PRAF (Programa de Asignación Familiar)

Honduras' PRAF, the oldest program in the genealogy (launched 1990), was restructured in 2000 to align with the standard CCT template. The restructured program conditioned transfers on school attendance and health visits, paid to the mother, targeted by PMT.

The PRAF evaluation produced a finding that no other program in the sample replicates with equal clarity. CCT increased incumbent mayor re-election by 39 percentage points. The mechanism is political capture: mayors who controlled beneficiary selection used the program to reward supporters and punish opponents. The transfer, nominally targeted to the poor, became an incumbent re-election tool. The finding documents a risk that the CCT template creates whenever the targeting function is controlled by local political authorities.

Brazil avoided this risk through self-declaration plus municipal validation (Fenwick, 2009); Honduras lacked the institutional capacity to replicate that safeguard. Nigeria's NASSP, where state-level implementation units control beneficiary selection at the LGA level, faces an analogous risk (Villatoro, 2005).

Ecuador: Bono de Desarrollo Humano (BDH)

Ecuador adopted the BDH in 2003 under the Gutiérrez administration. The program initially operated as a CCT with conditions on school attendance and health visits. In 2007, the Correa administration removed the conditions, converting the BDH to an unconditional transfer.

Schady and Araujo (2008) had documented that a significant share of BDH beneficiaries believed the transfer was unconditional even when conditions were formally in place. When the conditions were removed, enrollment did not fall. The finding is the information-channel effect: the behavioral impact of conditionality depends on whether beneficiaries know the conditions exist and believe they will be enforced. Where beneficiaries perceive the transfer as unconditional (because enforcement is invisible or because communication is poor), conditionality produces no behavioral effect regardless of its formal status. The BDH conversion showed that the distinction between CCT and UCT in practice depends on beneficiary perception as much as program design.

Chile: Solidario

Chile Solidario, launched in 2002, combined a cash transfer with a psychosocial support component. Families received a transfer while simultaneously being assigned a family-support worker who assisted with accessing social services, improving household Organization, and connecting to employment and training programs. The psychosocial component addresses a limitation of the standard CCT template: cash and conditions alone do not resolve the social exclusion, low self-efficacy, and information deficits that keep the poorest households from accessing services even when they are available and free. Chile Solidario is the hybrid model that the graduation literature (CGAP pilots in eight countries) later formalised as cash-plus-complementary. The evidence shows improved access to social services and reduced multidimensional poverty, but the relative contribution of the cash component versus the psychosocial component has not been cleanly isolated (Villatoro, 2005).

Peru: Juntos

Peru launched Juntos in 2005 under the García administration. The program targeted extreme-poverty households in rural areas with a transfer conditioned on school attendance, health visits, and attendance at nutrition sessions. Peru's implementation exposed a supply-side constraint sharper than any other Latin American case. In rural Andean communities, health facilities were distant and understaffed. The conditionality on health visits could be satisfied only by travelling to a facility that might be hours away, and the facility might lack a physician or a midwife on the day of arrival. The compliance burden fell on the household: the transfer required a visit that the supply side could not conveniently provide. Juntos continued operating but the conditionality enforcement was adapted to accommodate supply-side reality, moving toward the soft-conditional regime that Africa would later adopt as its default (Villatoro, 2005).

Bolivia: Bono Juancito Pinto

Bolivia's Bono Juancito Pinto, launched in 2006 under the Morales administration, adopted a universalist design: the transfer was paid to all families with children in public school, regardless of income. The universalist approach avoided the targeting costs and errors of PMT but diluted the fiscal impact on the poorest decile. The Bono's per-child transfer was small (approximately USD 28 per year) and its impact on poverty was correspondingly limited. The supply-side context was binding: rural schools in Bolivia lacked teachers, teaching materials, and infrastructure, and the Bono's conditionality on enrollment could not produce learning gains in schools that could not teach. The Bolivian case shows the instrument failing where both transfer adequacy and supply-side readiness are absent (Villatoro, 2005).

Uruguay: Asignaciones Familiares (AF)

Uruguay's AF, restructured in 2008, extended the existing social insurance system (contributory family allowances) with a non-contributory component for informal-sector households. The transfer was paid conditional on school attendance and health visits for children, with the amount varying by household composition. Uruguay's high institutional capacity (strong tax administration, universal civil registration, functional health and education systems) made CCT implementation straightforward: targeting was accurate, conditionality was enforced, and supply-side services were available. AF produced enrollment and health utilization gains consistent with the global evidence, without the implementation challenges that weaker-institution contexts faced. Uruguay is the confirmation case: where all four conditions (adequate transfer, functional supply-side, institutional permanence, evaluation infrastructure) hold, the instrument delivers its predicted effects (Villatoro, 2005).

Jamaica: PATH (Program of Advancement Through Health and Education)

Jamaica's PATH, launched in 2001, consolidated seven pre-existing cash and in-kind transfer programs into a single conditional cash transfer. The consolidation reduced administrative duplication but produced a program that was smaller in per-beneficiary value than several of its predecessors. PATH conditioned on school attendance and health visits, with the transfer paid bimonthly. The evaluation documented attendance and health utilization gains, but the transfer size (approximately USD 15 per household per month) was below the adequacy threshold for widespread welfare and productive effects. Jamaica's experience illustrates the fiscal constraint that small-island economies face: the administrative case for consolidation is strong, but consolidation without adequate transfer sizing produces a program that meets its administrative objectives without meeting its poverty-reduction objectives (Villatoro, 2005).

Three Patterns

The Latin American diffusion produces three patterns that the subsequent African and Asian evidence confirms and extends.

Adequate capacity produces predicted effects. Where institutional capacity is sufficient to enforce conditionality, monitor compliance, and deliver payments predictably (Colombia, Uruguay), the program produces the enrollment, health, and consumption effects the evidence base predicts. This is the confirmation case: the instrument works as designed when the conditions for its operation are met.

Weak capacity produces degraded outcomes. Where institutional capacity is insufficient to enforce conditionality at scale (Peru, Honduras, Ecuador), the effective design degrades toward soft-conditional or unconditional. The degradation preserves the income effect (consumption increases) while reducing or eliminating the compliance effect (enrollment gains are smaller, conditionality earns less of its administrative cost). This is the adaptation case: the instrument mutates in response to its environment, and the mutation changes what the instrument can claim to achieve.

Absent ownership produces program mortality. Where domestic political constituencies do not defend the program against partisan opposition (Nicaragua), the program dies regardless of its technical performance. Where local political actors capture the program for electoral purposes (Honduras), the program produces political effects alongside or instead of poverty-reduction effects. This is the political fragility case: the instrument's survival depends on institutional conditions that sit outside its design.

The three patterns predict the African and Nigerian experience. Africa enters the genealogy with weaker institutional capacity than Latin America, weaker supply-side infrastructure, and weaker political ownership of externally designed programs. The conditions for the instrument's success are lower on the continent, and the adaptation and mortality patterns are more likely to appear. Section 7 confirms this prediction.

Section 6. Asia Adoption

The Latin American CCT template reached Asia through the same World Bank technical-assistance channel that had carried it from Mexico City to Tegucigalpa. Three countries adopted the instrument between 2005 and 2007, each adapting it to a different institutional surface. The design features that travelled were the core logic (cash to mothers, conditioned on human-capital behavior, targeted by proxy means test) and the evaluation architecture (randomized or quasi-experimental assignment). The features that mutated were the conditionality breadth (India attached a single condition where Mexico attached three), the supply-side assumption (Philippine and Indonesian health infrastructure fell below the Latin American baseline), and the fiscal anchor (India funded JSY from general revenue at a scale that dwarfed the per-beneficiary transfer of any Latin American program).

Philippines: Pantawid Pamilyang Pilipino Program (4Ps)

President Gloria Macapagal-Arroyo launched 4Ps in 2007, supported by World Bank credit and technical design. The program imposed classical conditionalities: school enrollment and attendance of children aged 3 to 14, health check-ups for children aged 0 to 14, and attendance at family development sessions. The transfer ranged from USD 30 to 45 per household per month, paid to the mother. By 2020 the program covered 4.4 million households, making it the largest Asian CCT (World Bank, 2014).

The Philippine adoption tests two specific propositions. First, whether the Latin American template functions in an archipelagic state where transport costs to schools and clinics are higher than in the continental settings where the instrument was designed. Conditionality enforcement in remote barangays remained partial; the Department of Social Welfare and Development could verify attendance in urban and lowland areas but not reliably in upland and island communities. Second, whether the instrument produces the same distributional effect in a country with a less progressive pre-transfer income distribution. The Philippine Gini coefficient at program launch stood at 0.44, close to Brazil's 0.46 but applied to a per-capita income less than one-third of Brazil's. The transfer, at roughly 6 to 8 percent of beneficiary household consumption, sat below the 20 percent threshold that Bastagli et al. (2016) identify as associated with widespread productive and welfare effects.

School enrollment and health service utilization increased among beneficiaries (World Bank, 2014). The evidence on whether the program shifted long-term human capital is thinner. The World Bank's own impact evaluation documented attendance gains but noted that learning outcomes remained contingent on school quality, a supply-side variable the transfer does not address. Reyes et al. (2016) found that 4Ps improved food expenditure and reduced the probability of being poor, but the effect size on consumption was modest relative to the transfer share of income. The program also carried political risk: patronage dynamics in Philippine local politics created incentives for mayors to influence beneficiary selection, a risk documented in the Latin American experience by the Honduras PRAF electoral-capture finding (Villatoro, 2005).

Indonesia: Program Keluarga Harapan (PKH)

President Susilo Bambang Yudhoyono launched PKH in 2007 as a post-fuel-subsidy-reform social protection response, with World Bank technical support. PKH imposed conditionalities on maternal and child health (prenatal and postnatal visits, immunization), child education (enrollment and attendance for children aged 6 to 15), and nutrition monitoring. The transfer averaged USD 30 to 50 per household per month. By 2022 the program reached 10 million families (Kusuma et al., 2017).

PKH produced the strongest health-specific evidence of any Asian CCT. Kusuma et al. (2017) conducted a clustered randomized trial and found that after two years, PKH increased complete vaccination rates among children under two. This result matters because it extends the CCT evidence base beyond the schooling domain that dominates the Latin American literature. Vaccination is a health behavior with low recurrent cost to the household but high externalities; conditionality that moves a family from incomplete to complete vaccination schedule generates a public-health return that purely income-driven uptake might not achieve.

Indonesia's adoption also exposed a supply-side constraint sharper than any Latin American case. The 2012 World Bank supply-side readiness assessment found that 42 percent of community health centres in PKH target areas lacked a physician and 23 percent lacked a midwife. Where the facility is absent, the conditionality cannot be satisfied and the household is excluded from the transfer, a penalty that falls on the poorest. PKH addressed this by permitting soft compliance in underserved areas, effectively converting the CCT to a UCT in those sub-districts. The instrument mutated in transit: the conditionality that distinguished it from unconditional alternatives was relaxed where it could not function, producing a de facto hybrid.

India: Janani Suraksha Yojana (JSY)

The Government of India launched JSY in 2005 under the National Rural Health Mission. JSY is a single-condition CCT: a one-time cash payment of USD 8 to 17 per institutional delivery, with the amount varying by state and higher in designated low-performing states. By 2010 to 2011, the program reached over 11 million beneficiaries per year (Carvalho & Rokicki, 2019).

JSY is the most significant departure from the Latin American template in the Asian cohort. The classical CCT conditions on recurrent behavior (school attendance, health visits) and pays a periodic transfer. JSY conditions on a single event (institutional delivery) and pays a one-time transfer. The design narrows the human-capital claim from "simultaneous poverty reduction and investment in future human capital" to "removal of a specific financial barrier to a specific health service." The evidence on whether this narrower claim is met is affirmative on the proximate outcome and ambiguous on the distal one.

Lim et al. (2010), published in *The Lancet*, documented significant increases in institutional delivery and skilled birth attendance in low-performing states. Carvalho and Rokicki (2019) replicated the finding and confirmed robustness to model specification. Neonatal mortality declined in some states. The proximate claim, that paying for delivery increases delivery in facilities, is well supported. The distal claim, that institutional delivery reduces maternal mortality, is contested by evidence on the quality of care inside those facilities. Studies document overcrowding, absent personnel, and out-of-pocket costs that persist despite the transfer. The JSY transfer of USD 8 to 17 covers the official user fee but not the unofficial costs, transport, and income foregone that together exceed the transfer value in many states (Carvalho & Rokicki, 2019).

JSY's scale, over 10 million beneficiaries per year, exceeds any Latin American program in absolute terms. Its per-beneficiary cost, a one-time USD 8 to 17, is the lowest in the cohort. The ratio of scale to cost makes JSY the most fiscally efficient CCT in the sample and the one with the narrowest human-capital claim.

What travelled, what mutated

Three features of the Latin American design travelled intact. The core logic (cash to the female caregiver, conditioned on a specified behavior, targeted to the poor) was adopted in all three countries without modification. The evaluation architecture (randomized or quasi-experimental assignment) was embedded in 4Ps and PKH from the start; JSY received a post-hoc evaluation of sufficient quality for replication in a peer-reviewed journal. The delivery mechanism (payment to the mother) was preserved in all three cases.

Three features mutated. The breadth of conditionality narrowed from the Latin American triad (school, health, nutrition) to a single condition in JSY and a variable-conditionality regime in PKH where enforcement depended on supply availability. The supply-side assumption, that the services conditionality requires exist and are functional, broke down in Philippine remote areas and Indonesian underserved districts; the instrument adapted by softening enforcement, moving the effective design toward unconditional. The fiscal anchor shifted from the IDB/World Bank credit lines that financed Latin American programs to domestic general revenue (JSY) and a mix of domestic and IBRD financing (4Ps, PKH), with implications for sustainability that the Nigeria experience in Section 8 will make explicit.

The Asian cohort confirms a pattern the Latin American diffusion already suggested: the CCT instrument is a family of designs parameterised by conditionality breadth, enforcement strength, transfer adequacy, and supply-side readiness. The parameters that produce strong results in Mexico (high conditionality, adequate transfer, functional schools and clinics) produce weak or null results when transplanted without adaptation to

settings where the supply side cannot bear the demand the conditionality creates. Asia adapted the instrument by narrowing the conditionality (India), relaxing enforcement where supply failed (Indonesia), or accepting partial compliance as the operational norm (Philippines). Each adaptation moved the effective design closer to the unconditional end of the continuum.

Section 7. Africa Adoption

Sub-Saharan Africa adopted the cash transfer instrument later than Latin America and Asia, and under different conditions. Where Latin America had stabilised fiscal frameworks and functioning health and education systems to which conditionality could attach, and where Asia had at least partial supply-side infrastructure, much of SSA offered neither. The CCT instrument that arrived via World Bank and UNICEF technical assistance encountered health facilities that were absent or unstocked, schools without teachers, and administrative systems that could not monitor compliance at scale. The instrument mutated in response: conditionality weakened, enforcement softened, and the effective design of most African cash transfer programs moved toward unconditional or quasi-unconditional delivery. The mutation is not a defect of implementation; The mutation reflects the instrument adapting to its environment rather than a failure of implementation. Whether the adaptation preserves the human-capital rationale that distinguishes CCT from pure income support is the question this section addresses.

Ghana: Livelihood Empowerment Against Poverty (LEAP)

Ghana launched LEAP in 2008, making it one of Africa's earliest national cash transfer programs and the first to be subjected to a rigorous impact evaluation on the continent. The Department of Social Welfare, with DFID and World Bank support, designed LEAP as a soft-conditional program: conditions on school attendance and health visits existed on paper, but enforcement capacity was limited and compliance monitoring was inconsistent. LEAP 1000, added in 2015, targeted pregnant women and mothers of children under 15 months, with conditionality on antenatal care and immunization (Handa et al., 2016).

At program launch the transfer stood at GHc 8 to 15 per beneficiary per month, equivalent to USD 8 to 15 at the 2010 exchange rate. By 2016, inflation had eroded the real value to USD 5 to 9 per month. The transfer constituted 16 percent of baseline household consumption on average, with a median of 13 percent (de Groot, 2016). The UNICEF Transfer Project identifies 20 percent of consumption as the threshold above which impacts on wellbeing and productive activity become widespread; LEAP fell below this threshold throughout its operation. A 60 percent increase in transfer size would have been required to reach the 20 percent median share.

The UNICEF Innocenti evaluation found that LEAP raised consumption, decreased poverty, and increased antenatal care utilization. It found no effect on fertility and a reduction in intimate partner violence. It found no significant effect on stunting, consistent with the global pattern that cash alone, without complementary nutrition interventions, does not move anthropometric outcomes (Bastagli et al., 2016). The absence of a stunting effect reflects transfer inadequacy rather than program design failure. A transfer at 13 percent of consumption cannot plausibly alter the dietary intake required to change child growth trajectories.

LEAP's experience documents two Africa-specific findings. First, inflation erosion without indexation is the default condition of cash transfer programs that lack an automatic adjustment mechanism, affecting African programs as much as Latin American or Asian ones. Ghana's nominal quadrupling of the transfer between 2010 and 2016 produced only a doubling in real terms. Second, soft conditionality in an African context is a pragmatic adaptation, constrained by administrative capacity rather than chosen by design. Where the administrative system cannot verify school attendance or health visits at the frequency the conditionality requires, declaring the program conditional without enforcing it creates a messaging effect that may produce part of the behavioral change at lower administrative cost. Whether this is efficient or self-deceiving depends on the size of the enforcement effect, a question the Malawi experiment answers directly.

Kenya: Cash Transfer for Orphans and Vulnerable Children (CT-OVC)

Kenya piloted CT-OVC in 2004 and scaled it from 2007 with UNICEF and DFID support. The program targeted households caring for orphans and vulnerable children affected by the HIV/AIDS epidemic. It

operated as an officially unconditional transfer, paying approximately USD 20 per household per month to roughly 350,000 households by 2020 (Handa et al., 2014).

CT-OVC produced some of the strongest evidence in the African literature on the protective effects of unconditional cash. Handa et al. (2014) found that the transfer reduced the odds of sexual debut by 31 percent overall and 42 percent among females, reduced depressive symptoms by 24 percent, and increased food expenditure and school enrollment. These are outcomes that conditionality is supposed to produce by mandating behavior; CT-OVC produced them without mandating anything. The result is consistent with the income-effect channel: a household with more resources keeps children in school longer, delays sexual activity, and experiences better mental health, regardless of whether a program officer verifies attendance.

The Kenyan evidence does not establish that unconditional transfers are as effective as conditional ones for all outcomes. Baird, McIntosh, and Özler (2011) in Malawi found that conditional transfers produced enrollment effects more than twice as large as unconditional transfers. What the Kenyan evidence establishes is that unconditional transfers produce non-trivial effects on outcomes that conditionality targets, and that these effects appear even when the transfer is modest (USD 20 per month, roughly 20 percent of beneficiary consumption). The 20 percent threshold that LEAP fell below, CT-OVC approximated, and the results reflected the difference.

Malawi: Zomba Cash Transfer Experiment

The Zomba experiment, conducted by Baird, McIntosh, and Özler (2011) and published in the *Quarterly Journal of Economics*, is the most important single study for the question this brief addresses. It was the first randomized trial anywhere in the world to compare conditional and unconditional cash transfers in the same population with the same transfer size, using a three-arm design: CCT conditional on school attendance, UCT unconditional, and control. The target population was adolescent girls in the Zomba district of Malawi.

The findings are precise and they cut in both directions.

On schooling, the CCT outperformed the UCT. Both arms increased enrollment relative to control, but the UCT effect was only 43 percent as large as the CCT effect. The CCT also produced significantly higher attendance (days present) and significantly improved learning outcomes (English reading comprehension, mathematics, cognitive ability tests). The UCT showed no detectable learning gains. Conditionality, when enforced, moved the marginal child into school and kept her there long enough to learn.

On teenage pregnancy and marriage, the UCT outperformed the CCT. The UCT arm reduced pregnancy by 34 percent and marriage by 48 percent. The CCT had no significant impact on either outcome. The mechanism is compositional: the CCT excludes girls who drop out (they lose the transfer), and these drop-outs are precisely the girls at highest risk of early pregnancy and marriage. The UCT retains them, and the income effect delays the transition to early union that poverty accelerates.

The Zomba experiment identifies a design trade-off that no other study has documented with equal clarity. Conditionality improves human capital for compliers (girls who stay in school because the condition requires it) but denies transfers to non-compliers (girls who drop out and lose the payment). These non-compliers are the most vulnerable: poorer, less likely to be enrolled at baseline, at higher risk of the adverse outcomes that poverty produces. The CCT, by enforcing school attendance as a condition of payment, achieves its human-capital objective by excluding the worst-off from the transfer. The UCT, by paying regardless of school status, protects the worst-off at the cost of smaller enrollment gains.

Baird, McIntosh, and Özler (2011) derive the design implication: a blended program that conditions transfers for younger adolescents (where the enrollment effect is largest and the pregnancy risk is lower) and transitions to unconditional transfers for older teenagers (where the pregnancy and marriage risk is highest and the enrollment gain from conditionality is smaller) would minimize the trade-off. No program in the sample implements this blended design, although the Ghana LEAP 1000 age-targeting and the Indonesia PKH soft-compliance regime approximate elements of it.

Burkina Faso: Nahouri Cash Transfers Pilot Project

Akresh, de Walque, and Kazianga (2013) conducted a four-arm randomized trial in rural Burkina Faso: conditional cash transfers to fathers, conditional cash transfers to mothers, unconditional cash transfers to fathers, and unconditional cash transfers to mothers, plus control. The design permits isolation of the conditionality effect and the recipient-gender effect simultaneously.

The central finding refines the Zomba result. CCTs were significantly more effective than UCTs for marginal children: girls, younger children, and lower-ability children. CCTs led to enrollment increases of 20.3 percent for girls, 37.3 percent for younger children, and 36.2 percent for low-ability children relative to the control mean. UCTs either had no statistically significant impact on these groups or showed effects significantly smaller than the CCT. For non-marginal children (boys, older children, higher-ability children), CCTs and UCTs produced similar enrollment increases: 21.8 and 22.2 percent for boys, 27.0 and 28.5 percent for higher-ability children.

The marginal-child hypothesis resolves the apparent contradiction between the Zomba and the global evidence. Conditionality is not uniformly superior or inferior to unconditional design; its value depends on which children the program intends to reach. For children whom parents would enrol anyway (non-marginal children), the conditionality is inframarginal and the income effect alone produces the same result. For children whom parents would not enrol without the condition (marginal children), the conditionality is binding and the CCT outperforms the UCT. The administrative cost of monitoring compliance is justified only to the extent that the program targets marginal children; for non-marginal children, it is deadweight cost.

South Africa: Child Support Grant and HPTN 068

South Africa's Child Support Grant (CSG) is the world's largest unconditional child cash transfer, reaching 13 million children at approximately USD 25 per child per month. It operates as a near-universal program in an upper-middle-income country with functional payment infrastructure (SASSA) that most African states cannot replicate. The CSG provides the counterfactual against which CCT effects in Africa are measured.

Kilburn et al. (2020) evaluated HPTN 068, a randomized trial of a conditional cash transfer for young women in rural Mpumalanga, conditional on 80 percent school attendance. The CCT reduced multidimensional deprivation across six dimensions, including dimensions not directly targeted (economic, living standards). The CCT broke the simultaneous deprivation of school attendance and child labor. It did not, however, reduce HIV incidence, which was the trial's primary endpoint.

The South African evidence shows that conditionality can reduce multidimensional deprivation even in a context where an unconditional transfer of comparable size already exists. The CCT effect was not a simple income transfer effect; it was a compliance effect that altered the allocation of time (school versus work versus risk) in ways the unconditional CSG did not. But the institutional preconditions are demanding: SASSA's payment system, school attendance monitoring infrastructure, and a research budget that the national program could not sustain at scale. HPTN 068 is a research trial, not a policy template for replication in low-capacity settings.

Tanzania: Productive Social Safety Net (PSSN)

Tanzania launched PSSN in 2013 through TASAF with World Bank support. The program adopted a hybrid design: a cash transfer component plus a public works component, with soft conditions on school attendance and health visits. The program reached 1.1 million households by 2020, with transfers of USD 10 to 25 per household per month depending on the component mix (de Walque et al., 2012).

The PSSN design tests whether combining cash with productive inclusion (public works) produces outcomes that cash alone cannot. The evidence is mixed. The cash transfer component improved consumption and food security. The public works component provided temporary employment but produced assets of variable quality; the productive return on public-works investment in low-capacity local government settings is

uncertain. De Walque et al. (2012) also evaluated a CCT for HIV and STI prevention and found short-term behavior change that did not persist after payments stopped. The transient effect replicates a finding from multiple CCT-for-HIV trials: cash can shift behavior while the incentive is active, but the behavior reverts when the payment ends. Conditionality that targets a behavior the individual would not adopt without payment produces compliance during the program and attrition after it.

What Africa's adoption reveals

Five findings emerge from the African cohort that do not appear in the Latin American or Asian evidence.

First, conditionality attenuates toward unconditional in practice across most SSA programs. The CCT design arrives with conditionalities on paper; the administrative system cannot enforce them at scale; the effective design becomes UCT with messaging. This is the instrument encountering its supply-side dependency, which prevents the conditionality from functioning and forces the program toward the design that can. Where the school is absent, the clinic unstocked, and the compliance officer unpaid, the conditionality cannot function and the program converts to the design that can.

Second, the conditionality trade-off identified in Zomba is real and it is general. CCTs improve enrollment for marginal children at the cost of excluding the worst-off from the transfer. UCTs protect the worst-off at the cost of smaller enrollment effects. The trade-off is between two goods that cannot be simultaneously maximised. The blended design (conditional for younger adolescents, unconditional for older ones) that resolves the trade-off has not been implemented by any program in the sample.

Third, transfer adequacy, not conditionality, is the binding constraint in most SSA programs. LEAP at 13 percent of consumption, Ghana's flagship, fell below the threshold for widespread impacts. CT-OVC at 20 percent of consumption produced significant effects without conditionality. The size of the transfer predicts the size of the effect more reliably than the presence or absence of conditions. Bastagli et al. (2016) document this pattern globally; the African evidence confirms it with specific program-level data.

Fourth, the supply-side constraint is binding in a way that the Latin American experience did not reveal because Latin American supply-side capacity was higher at the point of CCT adoption. African health and education infrastructure was weaker in 2008 than Mexican infrastructure was in 1998. The CCT instrument assumes a service platform to which conditionality can attach; where the platform does not exist, the instrument cannot deliver its human-capital rationale.

Fifth, African cash transfer programs produce effects on outcomes that conditionality does not target: reduced intimate partner violence (LEAP), reduced depressive symptoms (CT-OVC), delayed sexual debut (CT-OVC), reduced multidimensional deprivation (HPTN 068). These are income-effect outcomes separate from compliance-effect outcomes, and they appear regardless of whether the program is conditional or unconditional. They constitute an empirically documented return to the transfer that the human-capital framing of CCT does not capture and that the poverty-reduction framing of UCT does.

Section 8. Nigeria 2007 to 2026: COPE, NASSP-HUP, Suspension, Renewed Hope

Nigeria encountered the cash transfer instrument through two channels simultaneously: the domestic political channel, which produced COPE in 2007, and the World Bank technical-assistance channel, which produced the National Social Safety Net Project (NASSP) in 2016. The two channels produced two different designs, two different accountability architectures, and two different records of performance. The distance between them measures the distance between a program designed for political visibility and one designed for results verification. Neither achieved the transfer adequacy or institutional permanence that the Latin American origin cases established as the minimum conditions for sustained impact.

COPE and SURE-P: The Domestic Pilots, 2007 to 2015

President Umaru Yar'Adua's administration launched the In Care of the People (COPE) program in 2007 with UNICEF technical support. COPE imposed classical conditionalities: school attendance and health visits for children in beneficiary households. The transfer ranged from USD 10 to 25 per household per month (NGN 1,500 to 5,000). At peak, COPE covered approximately 22,000 households across 12 states (Okoli et al., 2014).

The SURE-P Maternal and Child Health CCT pilot, launched under the Jonathan administration in 2012, targeted a narrower objective: increasing facility-based delivery by paying women up to NGN 5,000 (approximately USD 30) for completing antenatal care, skilled delivery, and postnatal care. The pilot operated in 37 primary health centres across 9 states and used geographic targeting, any woman attending a participating PHC was eligible, avoiding the administrative cost of household means-testing (Okoli et al., 2014).

Both programs closed without formal evaluation. COPE ended in 2015 with no published impact assessment. SURE-P was discontinued in the same year, and its design lessons were not incorporated into the NASSP-HUP framework that replaced it. The absence of evaluation means that the Nigerian government entered the NASSP design process in 2016 with no domestic evidence base on whether its own CCT pilots had achieved their objectives. The World Bank project appraisal drew instead on the Latin American and global evidence, applying findings from Mexico and Brazil to a context with substantially weaker supply-side infrastructure and administrative capacity.

NASSP-HUP: The World Bank Architecture, 2016 to 2022

The National Social Safety Net Project was approved by the World Bank Board in June 2016, financed by an IDA credit of USD 500 million plus the Government of Nigeria's contribution of USD 322.5 million from Abacha Restituted Funds. The project launched the regular cash transfer (regular-CT) program in all 36 states and the FCT, targeting poor households identified through the National Social Register (NSR) using a combination of geographic targeting, community-based targeting, and proxy means test (PMT) validation.

The transfer was set at NGN 5,000 per household per month. At the 2018 exchange rate, this was approximately USD 12.17, representing 6 to 7 percent of average monthly consumption of poor households as estimated from the 2018/19 NLSS (World Bank, 2024). This figure falls below the 13 percent median share that LEAP achieved in Ghana and far below the 20 percent threshold associated with widespread impacts. The Nigerian transfer, by design, was too small to produce the consumption and productive effects documented in the global evidence for adequately sized transfers.

The NSR, the program's targeting instrument, enrolled 10.1 million households by September 2021, covering approximately 20 percent of the national population. Of these, 8.4 million were in rural areas and 1.7 million in urban areas. The regular-CT program reached 1.8 million households by the same date, with 94 percent of beneficiaries female and 77 percent in rural areas (World Bank, 2024).

The IEG Implementation Completion and Results Review rated the project outcome as Satisfactory and efficacy as Substantial. The project exceeded its direct beneficiary target of 5 million, reaching 9.67 million. Electronic payment coverage reached 100 percent. The targeting accuracy, however, failed against its own benchmark: only 53 percent of beneficiaries fell in the bottom two consumption quintiles against a target of 80 percent. The co-responsibility (top-up conditional) component reached only 6 percent of beneficiaries against a target of 80 percent. The impact evaluation was not finalised at project close; the baseline had been conducted at midterm, a sequencing failure that left the project without a clean counterfactual estimate of program effect (IEG, 2024).

Implementation Failures on the Ground

The ICIR's 2018 investigation documented delivery failures that official reports acknowledge only in abstract terms. Beneficiaries in Niger and Oyo states received payments erratically: most received only six months of payments in a year while some community members received ten. Payment service providers contracted by the program had discrepancies in disbursement, leading to contract termination and a reversion to manual cash payments at Local Government Areas. Beneficiaries who missed the designated payment day forfeited the amount with no reclaim mechanism. Exclusion errors were documented: eligible households never received payments, and village heads confirmed irregularities in who was included. No grievance mechanism was accessible at the time of the investigation (ICIR, 2018).

The World Bank's June 2022 Implementation Status Report confirmed persistent disbursement delays. "Most of the complaints stem from delays in Cash Transfer payment," the report stated, noting the ongoing transition to fully digitised NUBAN accounts as a corrective. NASSCO's 2025 statement confirmed that the digital payment transition, tied to National Identification Number verification, had slowed disbursements but was necessary to eliminate ghost beneficiaries (NASSCO, 2025).

The NASSP Scale-Up PAD: Renewed Hope Architecture, 2024

The National Social Safety Net Program Scale-Up (NASSP-SU), approved in 2022 with an IDA credit of USD 800 million, restructures the program under the Tinubu administration's Renewed Hope framing. The project development objective is to "expand coverage of shock responsive safety net support among the poor and vulnerable and strengthen the national safety net delivery system" (World Bank, 2024).

The scale-up design comprises three components. Component 1, the Economic Shock Responsive Cash Transfer (ESR-CT), allocates USD 600 million to provide temporary transfers of NGN 5,000 per month for six months to 3.2 million rural households (through the NSR) and 5.0 million urban households (through the Rapid Response Register). Component 2, the Extended Regular Cash Transfer (ER-CT), allocates USD 147 million to extend transfers for a minimum of two years to the 1.8 million existing regular-CT beneficiaries and provide an exit grant. Component 3 allocates USD 53 million to delivery system strengthening (World Bank, 2024).

The transfer amount is unchanged at NGN 5,000 per month. Against a food inflation rate that reached 26 percent year-on-year in January 2025 and 21 percent in May 2025, the NGN 5,000 transfer has undergone the same erosion that afflicted LEAP in Ghana and N-Power in Nigeria. No indexation mechanism appears in the PAD. The design includes an inflation-adjustment clause in the annex ("the transfer value must be appropriately related to the project objectives and have the capacity to adjust, as necessary, to reflect inflation and cost increases"), but the operational transfer remains fixed at NGN 5,000 and the PAD provides no automatic trigger for adjustment.

The risk rating tells the design's own assessment of implementation probability. The overall project risk is Substantial, with five of nine risk categories rated Substantial: political and governance, macroeconomic, technical design, fiduciary, and environment and social. The political risk reflects the history of program suspensions and the federal-state coordination challenges that the NASSP experienced. The macroeconomic risk reflects Nigeria's inflation trajectory and fiscal constraints. The technical design risk reflects the scale of the urban expansion (5 million new households through the RRR, a novel targeting mechanism using

satellite imagery and USSD self-registration) and the absence of a prior implementation track record for the approach at this scale.

The January 2024 Suspension

The Presidency suspended all four National Social Investment Program (NSIP) components on 8 January 2024, pending a fraud investigation. The suspension was announced for six weeks; in the event it lasted approximately 12.5 months. The presidential panel's findings were not published. Over NGN 30 billion was recovered from the GEEP/GVG microcredit streams and frozen but not remitted to the Treasury Single Account, leaving programs unable to fully resume (Office of the SGF, 2024; Nigerian Tribune, 2025).

The suspension affected the cash transfer component indirectly. NASSCO and the NCTO, the federal implementing agencies, could not disburse during the suspension period. Beneficiaries who were already enrolled and receiving transfers experienced payment interruption. The FIJ reported in July 2024 that 400,000 Batch C Stream 2 N-Power beneficiaries were owed NGN 108 billion in arrears; while this figure pertains to the sibling N-Power program rather than the cash transfer, it illustrates the scale of arrears that accumulated across the NSIP portfolio during the suspension (FIJ, 2024; ANEEJ, 2025).

N-Power as Sibling Program: The Stipend Erosion Finding

N-Power, launched the day after the World Bank Board approved NASSP in June 2016, operated as the domestically financed component of the social investment portfolio. It shared NASSCO's delivery infrastructure but sat outside the World Bank's results framework and its accountability apparatus. The NGN 30,000 monthly stipend, set in 2016 and held nominally constant through 2026, underwent erosion that documents the consequences of omitting indexation in a high-inflation environment. Deflated by the NBS food sub-index, the 2016 stipend retained an estimated NGN 3,000 to 5,000 of its original purchasing power by 2026, roughly one-sixth to one-tenth of the launch value. Restoring 2016 real value would require a 2026 stipend on the order of NGN 180,000 to 300,000 (Mashood, 2026, Brief 4).

The N-Power payroll carried documented ghost beneficiaries at an estimated 9 to 18 percent of claimed reach, derived from investigative journalism estimates by Sahara Reporters and TheCable. The only published impact study, a cross-sectional survey of 99 Lagos beneficiaries, found a statistically significant but modest positive effect ($R^2 = 0.145$) and 41.4 percent of respondents still unemployed after program exit (Osimen et al., 2025). No national post-exit tracer study, no randomized or quasi-experimental evaluation, and no cost-effectiveness calculation was ever conducted (IPA & J-PAL, 2025).

The N-Power record matters for the cash transfer brief because it documents two failure modes that the NASSP-HUP cash transfer shares: the absence of an indexation mechanism (the NGN 5,000 transfer erodes by the same inflation that eroded the NGN 30,000 stipend), and the vulnerability of domestically financed programs to political suspension without accountability for the interruption (the January 2024 suspension froze disbursements across the portfolio for 12.5 months without published panel findings).

Nigeria's Record Against the Origin Cases

Mexico in 1997 and Brazil in 2003 established four conditions that the global evidence identifies as necessary for sustained cash transfer impact: adequate transfer size (at or above 20 percent of beneficiary consumption), functional supply-side infrastructure (schools and clinics that conditionality can reference), institutional permanence (program survival across political transitions), and evaluation infrastructure (randomized or quasi-experimental baseline and follow-up). Nigeria in 2026 meets none of these conditions fully.

The transfer at NGN 5,000 per month (6 to 7 percent of poor household consumption) is below the adequacy threshold. The supply-side infrastructure (primary health centres without physicians in 42 percent of target areas, pupil-teacher ratios above 40:1 in northern states) cannot support the conditionality the program nominally requires. Institutional permanence was interrupted by the January 2024 suspension, and the program's survival under the Renewed Hope framing rests on a PAD that has not yet completed its first

disbursement cycle. The impact evaluation, scheduled for the NASSP, was not completed at project close; the NASSP-SU PAD commits to an impact evaluation but has not yet reported results.

The distance between Nigeria's cash transfer in 2026 and Mexico's Progresa in 1997 is the distance the instrument travelled without the institutional conditions that made it work at origin. The instrument arrived; the conditions that give it its human-capital rationale did not.

Section 9. What the Pattern Shows

Eleven countries over twenty-nine years have now told their stories on the same five points. The pattern that emerges is a set of conditional findings: the cash transfer instrument produces reliable effects under some conditions, partial effects under others, and negligible or adverse effects under a third set. The conditions that distinguish the three categories are transfer adequacy, supply-side readiness, institutional durability, and the specific outcome the program targets. This section aggregates the country evidence, identifies what works reliably, what works sometimes, and what does not work, and presents the cross-program comparison in three charts.

What works reliably

Four outcomes appear with sufficient consistency and effect size across the evidence base to be treated as reliable returns to the cash transfer instrument.

Consumption and monetary poverty. Of 35 studies examining total expenditure in Bastagli et al. (2016), 25 find a statistically significant increase.. Of 31 studies on food expenditure, 23 find an increase. FGT poverty measures decline significantly in approximately two-thirds of studies. The effect is present in conditional and unconditional programs, in Latin America, Asia, and Africa, and at transfer levels both above and below the 20 percent threshold. The magnitude of the effect scales with the transfer size. This is the instrument's core function and its most robust finding: cash transfers increase the consumption of poor households and reduce the probability that those households fall below the poverty line.

School enrollment and attendance. Both CCTs and UCTs improve enrollment relative to no transfer. CCTs produce larger enrollment effects, especially when conditionality is explicitly monitored and enforced with penalties for non-compliance. The Bastagli et al. (2016) systematic review estimates approximately 60 percent improvement in the odds of enrollment for explicitly conditional programs with monitored compliance, relative to programs with no or weak conditions. The effect is present in Mexico (Parker & Vogl, 2018), Colombia, Nicaragua, the Philippines, Indonesia, and the African RCTs (Baird et al., 2011; Akresh et al., 2013). It is the human-capital return that justifies the conditionality premium.

Health service utilization. Cash transfers increase the uptake of preventive and routine health services: antenatal care visits, immunization , facility-based delivery, growth monitoring. The effect appears in both conditional and unconditional programs, though CCTs produce larger utilization effects where the health facility exists and is accessible. Indonesia's PKH increased complete vaccination rates (Kusuma et al., 2017). India's JSY increased institutional delivery (Lim et al., 2010; Carvalho & Rokicki, 2019). Ghana's LEAP increased antenatal care utilization (Handa et al., 2016). The effect operates through two channels: the income effect (more resources to afford transport and service fees) and the compliance effect (conditionality mandates the visit). Where the facility exists, both channels operate; where it does not, neither can.

Child labor reduction. Cash transfers consistently reduce child labor. Bastagli et al. (2016) report that child labor decreases across the evidence base, with no study finding a significant increase. The effect is a by-product of the school enrollment effect (children in school are not available for work) and the income effect (households with more cash income need child wages less). It does not require conditionality; UCTs produce the effect at smaller magnitude.

What works sometimes

Three outcomes appear in the evidence but with effect sizes and significance levels that depend on design parameters or context.

Learning outcomes. School enrollment increases reliably; learning quality does not. The evidence on test scores is "small at best" (Bastagli et al., 2016). Mexico's Progresa produced enrollment gains of 12 percent at secondary level alongside modest improvements in test scores. The Philippines' 4Ps increased attendance without documented learning gains. The mechanism that moves a child from out-of-school to in-school is the

conditionality or the income effect; the mechanism that produces learning inside the school is the quality of instruction, which the transfer does not alter. Cash transfers solve the access problem without solving the quality problem.

Anthropometric outcomes (stunting, wasting, underweight). Dietary diversity and food security improve; child height and weight do not move consistently. Stunting effects are significant in 5 of 13 studies; wasting in 1 of 5; underweight in 1 of 8 (Bastagli et al., 2016). The result replicates in Africa: LEAP found no significant stunting effect; CT-OVC found nutrition improvements without anthropometric change. The explanation is biological: changing a child's growth trajectory requires sustained adequate intake during the critical window (conception to age 2), and the transfer size and duration in most programs are insufficient for this. Mexico's Progresa, which did produce birthweight gains (Barber & Gertler, 2008), operated with a transfer approximately four times the Nigerian level as a share of consumption.

Employment and productive activity. Adult labor supply effects are mixed: more than half of studies find no significant impact, and where significant, the majority find an increase rather than the work-disincentive effect critics allege. Savings, livestock ownership, and agricultural input investment increase consistently (Bastagli et al., 2016), even though no program explicitly targets these outcomes. The productive effects are income-effect by-products, not conditionality-driven, and they appear at transfer sizes above the 20 percent threshold. Below that threshold, the transfer is consumed rather than invested.

What does not work

Three failure modes appear with sufficient consistency to be treated as structural limitations of the instrument.

Conditionality without supply. Where the school, clinic, or immunization point that conditionality requires does not exist or is non-functional, the conditionality cannot produce the intended human-capital gain and may exclude the poorest households from the transfer. The failure appears in Bolivia (Bono Juancito Pinto, universalist alternative adopted because CCT targeting could not function with weak supply-side), Indonesia (PKH softened to de facto UCT in underserved districts), the Philippines (partial compliance in remote areas), and across SSA (LEAP, CT-OVC, and PSSN all operate with weaker conditionality than their designs specify). Heimo and Syvatera (2023) show that the World Bank and IADB discursively constructed the CCT narrative while concealing their role in promoting an instrument whose supply-side dependency they did not verify in the contexts of adoption.

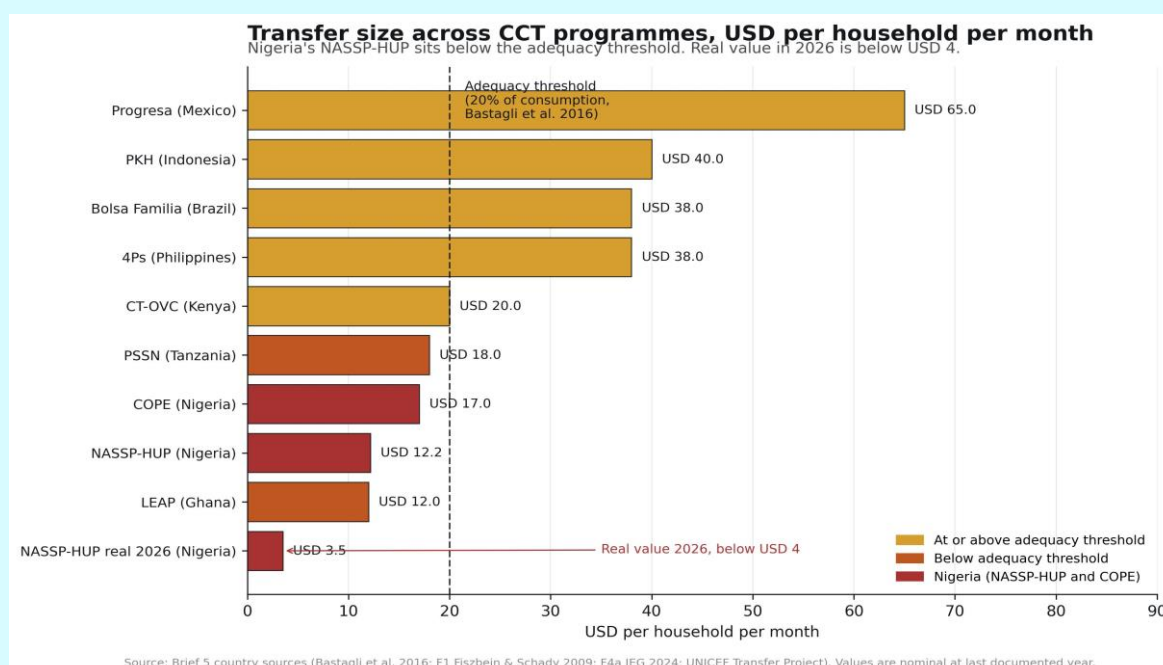
Transfer inadequacy. A transfer below 20 percent of beneficiary consumption produces consumption and food expenditure gains without the wider welfare, productive, and protective effects that larger transfers achieve. LEAP at 13 percent of consumption found consumption and poverty reduction but no stunting effect. Nigeria's NASSP at 6 to 7 percent of consumption has produced no documented impact on any outcome beyond the immediate consumption measure, and the impact evaluation to measure this was not completed at project close. The transfer adequacy finding is the single most replicated result in the evidence base: the effect size of a cash transfer is, within bounds, a function of the transfer size relative to the household's pre-transfer consumption.

Political fragility. Cash transfers that depend on a single administration's political commitment die with that administration. Nicaragua's RPS, internationally acclaimed, closed when the domestic constituency evaporated. Honduras's PRAF became an incumbent re-election tool. Mexico's Progresa was renamed twice (Oportunidades, Prospera) and replaced under AMLO with an unconditional pension. Nigeria's COPE closed without evaluation. The January 2024 NSIP suspension interrupted cash transfers for 12.5 months without published justification. Institutional durability requires either constitutional or legislative anchoring (Brazil's Bolsa Familia survived four administrations) or a delivery system that is technically independent of political appointment (NASSCO's institutional structure was designed for this but proved vulnerable to the 2024 suspension).

Table 1 & Chart 1:

Transfer Size in USD per Household per Month Across Programs

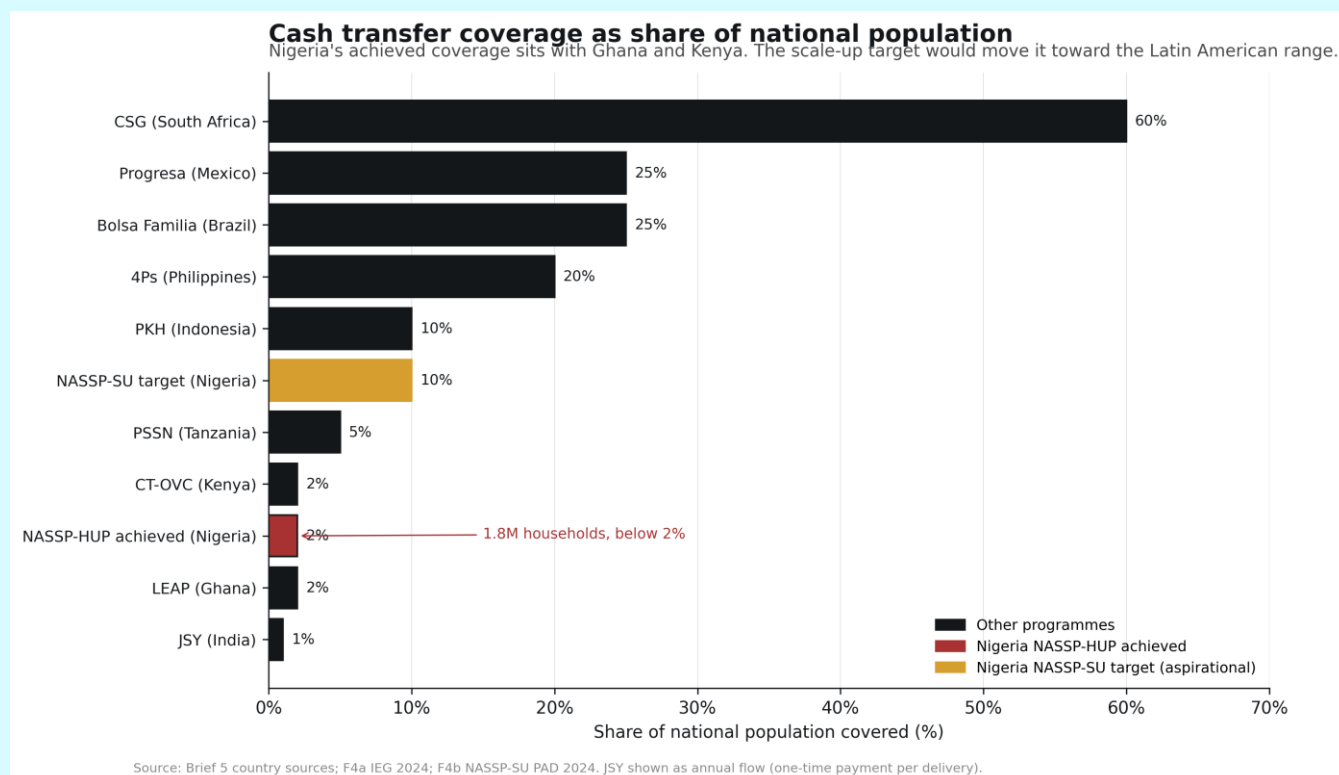
Program	Country	Year	USD/HH/month	% of consumption
Progresa	Mexico	1997	40-90	20-29%
Bolsa Familia	Brazil	2003	20-45	~17%
Familias en Accion	Colombia	2000	25-50	~17%
RPS	Nicaragua	2000	15-40	~29%
PRAF	Honduras	1990	10-20	~6%
4Ps	Philippines	2007	30-45	6-8%
PKH	Indonesia	2007	30-50	~15%
JSY	India	2005	8-17 (one-time)	n/a
LEAP	Ghana	2008	5-9 (2016)	13% median
CT-OVC	Kenya	2007	20	~20%
CSG	South Africa	1998	25	~15%
PSSN	Tanzania	2013	10-25	~10-15%
COPE	Nigeria	2007	10-25	n/a
NASSP regular-CT	Nigeria	2016	12.17	6-7%
NASSP-SU ESR-CT	Nigeria	2022	12.17 (nominal)	<5% (2026 real)



The chart shows the Nigeria transfer at the bottom of the distribution, below the adequacy threshold that the evidence identifies. The gap between Nigeria's NGN 5,000 and the transfer levels that produce documented productive and protective effects (Kenya's USD 20, Mexico's USD 40 to 90) is not marginal; it is an order of magnitude.

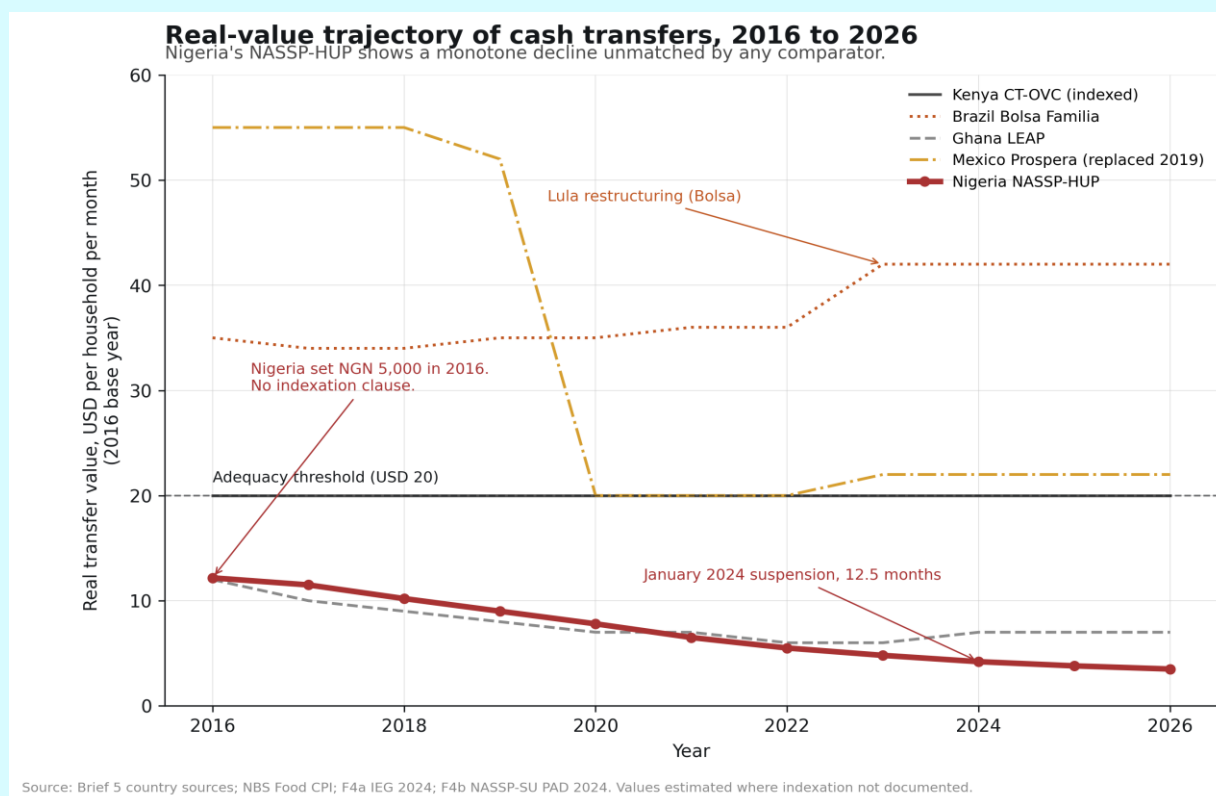
Table 2 & Chart 2: Coverage as Share of Population

Program	Country	HH covered	Population share
Progresa	Mexico	6.5M	~25%
Bolsa Familia	Brazil	13.9M	~25%
4Ps	Philippines	4.4M	~20%
PKH	Indonesia	10M	~10%
JSY	India	10M+/yr	<1% (annual flow)
LEAP	Ghana	116K	<2%
CT-OVC	Kenya	350K	<2%
CSG	South Africa	13M children	~60% (children)
PSSN	Tanzania	1.1M	~5%
NASSP regular-CT	Nigeria	1.8M	<2%
NASSP-SU target	Nigeria	10.2M	~10% (target)



Nigeria's achieved coverage under the regular-CT program is below 2 percent of the population, comparable to Ghana and Kenya. The NASSP-SU target of 10.2 million households (approximately 10 percent of the population) would bring Nigeria closer to the Latin American coverage range but remains an aspirational target rather than an achieved result.

Chart 3: Nigeria Real-Value Trajectory versus Comparators



The NGN 5,000 monthly transfer was set in 2016. Against the NBS food CPI, its real value in 2026 is estimated at less than one-third of its 2016 purchasing power. In the same period, Kenya's CT-OVC maintained its real value through periodic adjustment. Brazil's Bolsa Familia adjusted the basic benefit upward in 2023 under Lula's restructuring. Mexico's Prospera transfer was replaced by an unconditional pension nominally larger but eroded by the same inflation that weakened Prospera. Ghana's LEAP nominally quadrupled but only doubled in real terms.

Nigeria's cash transfer has no indexation mechanism. The NGN 5,000 that represented USD 12.17 at appraisal represents less than USD 4 at the 2026 exchange rate, and its food-purchasing power has fallen correspondingly. The trajectory is a monotone decline in real value, unmatched by any comparator in the sample. No program that allows its transfer to erode below the adequacy threshold has produced the wider welfare and productive effects that justify the instrument.

Section 10. CCT by Outcome Type: Where the Instrument Works Most

The country stories and the aggregate pattern establish that cash transfers produce different effects depending on the outcome domain targeted. This section classifies the 11 country programs plus Nigeria by their primary outcome type, measures the strength of evidence in each domain, and identifies where the instrument delivers its largest and most reliable returns. The classification answers a question the previous sections raised but did not resolve: is CCT an education instrument, a health instrument, a nutrition instrument, or an economic empowerment instrument? The answer determines what Nigeria should reasonably expect from its own program.

Classification Framework

Each program is classified by its primary conditionality domain (the behavior the transfer requires) and its documented strongest outcome (the domain where the effect size is largest and most consistently replicated). Where the conditionality spans multiple domains, the program appears in each with its domain-specific result.

Four outcome types:

- **Education access.** Conditionality on school enrollment and/or attendance. Measured outcome: enrollment rate, attendance rate, grade progression, drop-out reduction.
- **Health utilization.** Conditionality on health service use (prenatal visits, immunization , facility delivery, growth monitoring). Measured outcome: service uptake rate, vaccination completion, institutional delivery rate.
- **Nutrition.** Conditionality on nutrition-related behavior (dietary diversity, micronutrient supplementation, growth monitoring). Measured outcome: dietary diversity, food security, stunting, wasting, underweight.
- **Economic empowerment.** No conditionality on human-capital behavior; transfer targets income, consumption, or asset accumulation. Measured outcome: consumption, poverty headcount, savings, asset ownership, employment, income.

Evidence by Outcome Type

Table 3: Education Access CCTs

Program	Country	Condition	Enrollment effect	Learning effect	Evidence strength
Progresa	Mexico	School attendance 85%	+12% secondary	Modest	Strong (RCT, 20+ yr follow-up)
Bolsa Familia	Brazil	School attendance (soft)	Increased	Limited	Moderate (quasi-experimental)
Familias en Accion	Colombia	School attendance	+7-10%	Small	Strong (RCT)
RPS	Nicaragua	School + health	+13% rural	n/a	Strong (RCT, short-lived)
4Ps	Philippines	School + health + nutrition	Increased	Limited	Moderate
PKH	Indonesia	Education + health + immunization	Increased	n/a	Moderate
LEAP	Ghana	School + health (soft)	Increased	n/a	Moderate (RDD)
Zomba CCT	Malawi	School attendance	+43% relative to UCT	Significant	Strong (3-arm RCT)
Burkina Faso CCT	Burkina Faso	School attendance 90%	+20% girls, +37% younger	n/a	Strong (4-arm RCT)

Finding. Education-access CCTs produce the most consistent and largest effect sizes in the evidence base. Enrollment increases of 7 to 43 percent are documented across nine programs. The effect is largest where

conditionality is explicitly enforced (Malawi Zomba, Burkina Faso) and where the transfer is adequate relative to consumption. Learning effects are consistently small or absent; the instrument moves children into school without improving what happens inside the classroom.

Mechanism. The education-access effect operates through two channels. The compliance channel: conditionality mandates attendance and the household complies to retain the transfer. The income channel: additional resources reduce the opportunity cost of schooling (foregone child labor, transport costs, school fees). Baird, McIntosh, and Özler (2011) decompose these channels: the CCT compliance effect is 2.3 times the UCT income effect for school enrollment. Conditionality earns its administrative cost in the education domain, a finding confirmed by Akresh, de Walque, and Kazianga (2013) for marginal children.

Table 4: Health Utilization CCTs

Program	Country	Condition	Utilization effect	Health outcome	Evidence strength
Progresa	Mexico	Health visits	+18% clinic visits	+127g birthweight	Strong (RCT)
PKH	Indonesia	Maternal health + immunization	Increased complete vaccination	n/a	Strong (clustered RCT)
JSY	India	Institutional delivery	Significant increase	Neonatal mortality ↓ some states	Strong (Lancet + replication)
LEAP 1000	Ghana	ANC + immunization	Increased ANC	n/a	Moderate (RDD)
CT-OVC	Kenya	None (unconditional)	Increased health service use	↓ sexual debut 31%	Moderate
HPTN 068	South Africa	School attendance 80%	n/a	↓ multidimensional deprivation	Strong (RCT)

Finding. Health-utilization CCTs produce significant service-uptake gains, especially for immunization (PKH), facility delivery (JSY), and antenatal care (LEAP 1000). The effect operates where the health facility exists and is accessible. JSY's evidence on institutional delivery is among the strongest in the global CCT literature: the Lancet finding was replicated by Carvalho and Rokicki (2019). Progresa's birthweight gain of 127.3g is the only documented anthropometric effect at birth in the sample.

Mechanism. The health-utilization effect, like the education-access effect, operates through compliance and income channels. The compliance channel is particularly effective for discrete health behaviors with a single compliance point (one delivery, one vaccination series) because monitoring cost is low and the behavioral change is verifiable. This is why JSY, a single-condition CCT, produces a larger and more reliably replicated effect than multi-condition programs where monitoring each condition is more expensive.

Table 5: Nutrition CCTs

Program	Country	Nutrition condition	Dietary diversity	Stunting	Wasting	Evidence strength
Progresa	Mexico	Nutrition supplement	Improved	Reduced (short-term)	n/a	Strong
Bolsa Familia	Brazil	Health visits (implicit)	Improved	n/a	n/a	Moderate
4Ps	Philippines	Family development sessions	Improved	n/a	n/a	Moderate
LEAP	Ghana	Health visits (soft)	n/a	No effect	n/a	Moderate
CT-OVC	Kenya	None	Improved	No effect	No effect	Moderate

Finding. Nutrition CCTs improve dietary diversity and food security consistently. Anthropometric outcomes (stunting, wasting, underweight) do not move. The Bastagli et al. (2016) global synthesis confirms: stunting effects significant in 5 of 13 studies, wasting in 1 of 5, underweight in 1 of 8. Cash transfers improve what children eat without reliably changing how they grow. The gap reflects the biological requirement for sustained adequate intake during the critical window (conception to age 2), which most program transfers and durations cannot meet.

Mechanism. The nutrition effect is an income effect, not a compliance effect. Additional resources improve dietary diversity and food expenditure regardless of whether the program conditions on nutrition behavior. The anthropometric failure reflects transfer inadequacy (most transfers are too small to fund the sustained diet required for growth trajectory change) and duration inadequacy (most programs are shorter than the 1,000-day critical window).

Table 6: Economic Empowerment CCTs

Program	Country	Type	Consumption effect	Employment effect	Asset effect	Evidence strength
Bolsa Familia	Brazil	Soft conditional	Increased	Debated (no disincentive)	n/a	Moderate
LEAP	Ghana	Soft conditional	Increased, ↓ poverty	n/a	n/a	Moderate
CT-OVC	Kenya	Unconditional	Increased	n/a	n/a	Moderate
CSG	South Africa	Unconditional	Increased	n/a	n/a	Moderate
NASSP regular-CT	Nigeria	Soft conditional	Not measured	n/a	n/a	Absent

Finding. Economic empowerment effects are the weakest domain in the CCT evidence base, not because transfers fail to increase consumption (they do, reliably) but because "economic empowerment" in the strict sense (sustained income increase, employment creation, asset accumulation, exit from poverty) requires more than a cash transfer. Bastagli et al. (2016) find that savings and asset ownership increase spontaneously, even without program targeting, but these are by-products of adequate transfers, not independent mechanisms. No CCT program in the sample has demonstrated that beneficiaries exit poverty at scale after the transfer ends. Mexico's long-term evidence (Parker & Vogl, 2018) shows improved adult outcomes for women exposed as children, but the mechanism is human capital accumulation (more schooling leading to better jobs), not the transfer itself generating employment.

Mechanism. The consumption effect is immediate and mechanical: the transfer adds to household resources. The exit-from-poverty effect requires complementary interventions (productive inclusion, labor market access, financial services) that sit outside the cash transfer design. The CGAP graduation pilots, which combined cash with assets, training, and savings, produced earnings increases of 33 percent and consumption increases of 5 percent two years after program end, with ROI from 120 percent (Ghana) to 420 percent (India). These are the results of cash-plus-complementary, not cash alone.

Where CCT Works Most: The Hierarchy

Ranking the four outcome domains by evidence strength, effect size, and replication frequency:

- 1. Education access (strongest).** Largest effect sizes. Most consistent replication. Conditionality earns its cost. The CCT instrument's strongest validated purpose is increasing school enrollment and attendance among children who would otherwise be excluded.
- 2. Health utilization (strong).** Significant service-uptake gains. Particularly effective for discrete behaviors (immunization, facility delivery) where compliance is verifiable at low monitoring cost. The instrument's second validated purpose is increasing use of preventive and routine health services.
- 3. Nutrition (moderate).** Dietary diversity improves; anthropometry does not. The income effect improves what children eat. Transfer inadequacy prevents the instrument from changing how children grow. Nutrition is a by-product of adequate income, not a primary CCT mechanism.
- 4. Economic empowerment (weakest).** Consumption increases mechanically; sustained income and employment gains require complementary interventions. No CCT alone has demonstrated poverty exit at scale. The instrument smooths consumption without generating the productive capacity that would make the smoothing unnecessary.

Nigeria's Position Against the Hierarchy

Nigeria's NASSP-HUP conditions on school attendance and health visits, placing its formal design in domains 1 and 2 (education access and health utilization), where the global evidence is strongest. In principle, Nigeria has adopted the instrument for the purposes where it works most.

In practice, three deficits prevent the instrument from delivering on its chosen domains.

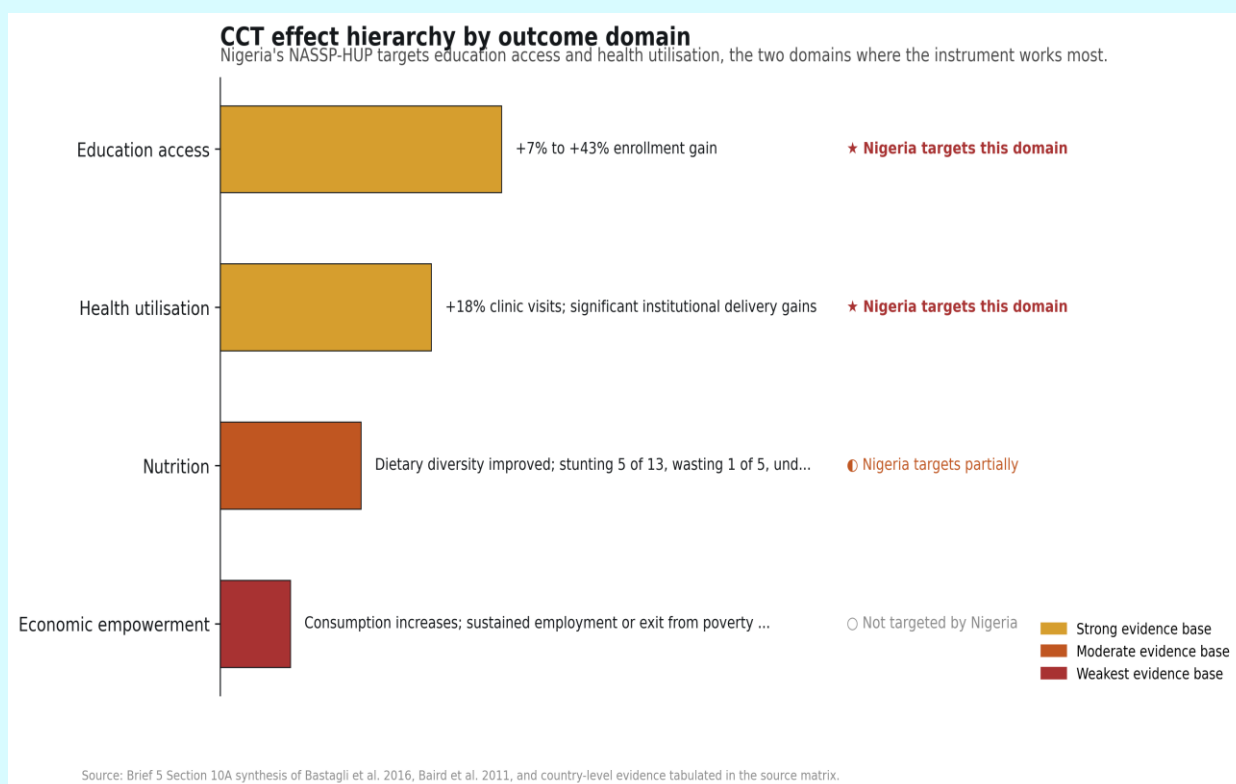
Transfer inadequacy. The evidence hierarchy shows that education-access effects require transfers at or above 20 percent of consumption. Nigeria's transfer at 6 to 7 percent of consumption falls below the threshold for both education and health effects. The global evidence predicts enrollment and utilization

effects at this transfer level; it does not predict the larger effects that would justify the program as a national education or health strategy.

Supply-side absence. Education-access effects require functional schools. Health-utilization effects require functional clinics. Nigeria's primary education infrastructure in the north (pupil-teacher ratios above 40:1, multiple states) and primary health infrastructure (42 percent of PHCs without a physician) cannot support the compliance channel. The conditionality mandates attendance and health visits; the facility shortage makes compliance impossible for the poorest households, who are also the least served.

Evaluation absence. The effect of Nigeria's CCT on enrollment and health utilization has never been measured. The IEG rated M&E as Substantial in process, but the impact evaluation was not completed at project close. The country has adopted the instrument for its strongest-evidence domains without generating the evidence that would confirm whether those effects materialise in the Nigerian context.

Chart 4: CCT Hierarchy by outcome domain



The Core Finding

CCT is an instrument for access and utilization, not for economic transformation. Its strongest evidence base supports two claims: conditional cash increases school enrollment, and conditional cash increases health service use. Both claims are conditional on adequate transfer size and functional supply-side infrastructure. The instrument does not contain a mechanism for sustained employment creation, structural poverty reduction, or economic empowerment beyond the transfer period. Programs that claim these broader outcomes (Bolsa Familia's poverty-reduction mandate, NASSP-HUP's objective of lifting 100 million out of poverty) are claiming results the instrument can support only with complementary interventions that sit outside its design.

Nigeria's program targets education access and health utilization, the domains where CCT works most. It operates below the transfer threshold and in the absence of the supply-side infrastructure that the evidence requires for these effects to appear. The instrument has been adopted for the right purposes but deployed under the wrong conditions. Section 11 specifies the conditions that would align deployment with purpose.

Section 11. Verdict on Cash as National Response

The question this brief addresses admits a conditional answer. Cash transfer is a sound national response to poverty for the specific objectives of consumption smoothing, monetary poverty reduction, and increased utilization of health and education services, provided that four conditions are met: the transfer is adequate (at or above 20 percent of beneficiary household consumption), the supply-side services that conditionality references are functional, the institutional architecture survives political transition, and the program is evaluated with a design capable of isolating program effect from selection effect.

Cash transfer is an unsound national response for the objectives of learning quality improvement, child growth trajectory change, sustained employment creation, or structural poverty reduction, because the instrument lacks mechanisms that produce these outcomes. These objectives require complementary interventions (school quality reform, nutrition supplementation, labor market policy, productive inclusion) that cash transfers complement without replacing.

The verdict is structured in three parts: what the instrument earns, what it costs, and where the balance falls for Nigeria.

What the instrument earns

The evidence from 11 countries and 201 studies (Bastagli et al., 2016) establishes that cash transfers produce five reliable returns. Consumption increases in 71 percent of studies. Food expenditure increases in 74 percent. Poverty measures decline in approximately two-thirds. School enrollment increases, with the CCT compliance effect adding approximately 60 percent to the odds of enrollment relative to weak or no conditionality. Health service utilization increases, with the effect present in both conditional and unconditional designs but larger under enforced conditionality.

These returns accrue at the household level and they are immediate. They are not contingent on long-term behavioral change; they are the direct consequence of transferring purchasing power to households that were previously below the consumption floor. The returns are also additive: the income effect produces consumption, nutrition, and protective effects (reduced IPV, reduced depressive symptoms, delayed sexual debut) that the human-capital framing of CCT does not explicitly claim, though the evidence documents them. The conditionality effect produces enrollment and utilization gains that the income effect alone does not achieve at the same magnitude.

The long-term returns are documented in one country. Parker and Vogl (2018) find that childhood exposure to Mexico's Progres a improves women's educational attainment, geographic mobility, labor market performance, and household living standards in early adulthood. For men, the effects are smaller. The long-term evidence is limited to one program in one institutional context; whether it generalises to programs with weaker transfers, weaker supply-sides, and shorter durations is not established.

What the instrument costs

The costs are of three kinds: fiscal, administrative, and opportunity.

The fiscal cost of cash transfers in the sample ranges from 0.08 percent of GDP (Paraguay, Chile) to 0.50 percent (Brazil, Mexico). Nigeria's NASSP-HUP at USD 800 million over three years, against a GDP of approximately USD 400 billion, represents approximately 0.07 percent of GDP annually, below the lower bound of the Latin American range. The fiscal cost is not the binding constraint; the transfer inadequacy is. Nigeria spends a smaller share of GDP on cash transfers than any Latin American comparator and receives proportionally smaller returns.

The administrative cost of conditionality is the monitoring and compliance-verification expenditure required to distinguish a CCT from a UCT. This cost includes school attendance tracking, health visit verification, management information systems for compliance, and the penalty administration for non-compliance. The evidence does not provide a systematic comparison of administrative cost ratios across CCT and UCT

programs. The Akresh, de Walque, and Kazianga (2013) finding that conditionality is inframarginal for non-marginal children implies that the administrative cost of monitoring compliance for children who would enrol anyway is a deadweight expenditure. The administrative cost is justified only to the extent that the program targets marginal children.

The opportunity cost is the alternative use of the fiscal and administrative resources. The Latin American evidence shows that cash transfers displaced food subsidies (Mexico), in-kind welfare programs (Brazil), and social funds (multiple countries). Section 2 identified four displaced alternatives: food subsidies, workfare, universal social insurance, and social funds. The opportunity cost question is whether the resources allocated to cash transfer would produce larger poverty-reduction returns if allocated to an alternative instrument. Aggregate comparison does not settle the question. The evidence is decisive at the level of specific objectives.. For consumption smoothing and utilization, cash dominates the alternatives. For employment, workfare or productive inclusion dominates cash. For structural poverty reduction, cash plus complementary services dominates cash alone.

Where the balance falls for Nigeria

Nigeria's cash transfer program in 2026 fails three of the four conditions that the evidence identifies as necessary for the instrument to earn its returns.

The transfer is inadequate. NGN 5,000 per month at 6 to 7 percent of poor household consumption is below the 20 percent threshold. The program cannot produce the wider welfare and productive effects that adequately sized transfers achieve, and the evidence from LEAP, CT-OVC, and the global synthesis confirms that effect size scales with transfer adequacy. The transfer has eroded since 2016 without indexation and will continue to erode under Nigeria's inflation trajectory.

The supply-side is non-functional in the areas where the program operates. The PAD itself rates technical design risk as Substantial. Primary health centres in target areas lack physicians and midwives. Pupil-teacher ratios in northern states exceed levels at which the enrollment gain from conditionality can plausibly translate into learning. The co-responsibility component reached 6 percent of beneficiaries against an 80 percent target, a failure rate that documents the gap between conditionality on paper and conditionality in practice.

Institutional durability is unproven. The January 2024 suspension interrupted transfers for 12.5 months without published justification or panel findings. The NASSCO structure designed for independence proved vulnerable to political intervention. The Renewed Hope framing is a PAD commitment, not an institutional track record. Brazil's Bolsa Familia survived four administrations because it built a decentralised delivery system (Cadastro Unico, 5,564 municipalities) that could not be dismantled by executive order. Nigeria's delivery system, at seven years of operation and one suspension, has not demonstrated equivalent resilience.

Evaluation infrastructure is absent. The NASSP impact evaluation was not completed at project close. The targeting accuracy failure (53 percent in the bottom two quintiles against an 80 percent target) was documented by the IEG but not remediated in the scale-up design. No randomized or quasi-experimental evaluation of the Nigerian cash transfer exists. The country that most needs rigorous evidence on whether its largest social protection instrument works is the country that has not generated that evidence.

The balance for Nigeria is therefore negative on current design parameters but positive on potential. The instrument can produce the returns the evidence documents if the transfer is raised to adequacy, the supply-side is addressed as a precondition rather than an afterthought, institutional durability is legislatively anchored, and evaluation infrastructure is built into the program from inception. The failure lies in the implementation conditions, which deny the instrument the environment in which it earns its returns. Section 11 specifies the design changes required to bring Nigeria's cash transfer from its current position below the adequacy threshold to a position where the evidence predicts impact.

Section 12. Recommendations for Nigeria's Cash Transfer Architecture

Seven recommendations follow from the pattern established in Section 9 and the verdict in Section 10. Each names the design parameter to change, the evidence that justifies the change, the specific action required, and the institutional actor responsible. The recommendations are ordered by their predicted impact on program effectiveness: the first three address the conditions that the evidence identifies as necessary for any impact; the last four address the conditions that determine whether the impact is sustained, measured, and protected from political disruption.

R1. Raise the transfer to adequacy and index it

Current position: NGN 5,000 per month, 6 to 7 percent of poor household consumption, below the 20 percent threshold.

Required position: NGN 15,000 to 20,000 per month at 2024 prices, equivalent to 15 to 20 percent of poor household consumption, with automatic semi-annual indexation to the NBS food CPI.

Evidence: Bastagli et al. (2016) document that transfers above 20 percent of consumption produce widespread welfare, productive, and protective effects; transfers below this threshold produce consumption and food expenditure gains without the wider returns. LEAP at 13 percent found consumption and poverty effects but no stunting impact; CT-OVC at approximately 20 percent found significant protective effects without conditionality. The threshold is a zone rather than a point estimate, and the current Nigerian transfer falls below its lower bound.

Indexation mechanism: The NASSP-SU PAD contains an inflation-adjustment clause in the annex but no operational trigger. The indexation should be made automatic and binding: a semi-annual review by an independent technical committee (not the implementing ministry) that adjusts the nominal transfer by the cumulative food CPI change since the last adjustment, with a floor of zero adjustment (no nominal cuts during deflation) and no cap (full inflation pass-through). The companion N-Power stipend documents the consequence of omitting this mechanism: a transfer that buys one-sixth to one-tenth of its launch value after a decade of inflation (Mashood, 2026, Brief 4).

Fiscal implication: At NGN 15,000 per month for 10.2 million households (the NASSP-SU target), the annual transfer cost is NGN 1.83 trillion, approximately 1.0 percent of Nigeria's 2024 GDP. This is above the current 0.07 percent but within the Latin American fiscal range (0.08 to 0.50 percent of GDP) and below Brazil's 0.50 percent. The fiscal space exists at the level of GDP; the constraint is budgetary allocation and the political economy of redirecting expenditure from subsidies to social protection.

R2. Address supply-side readiness as a precondition, not an afterthought

Current position: Co-responsibility component reached 6 percent of beneficiaries against an 80 percent target. Primary health centres in target areas lack physicians and midwives. Conditionality on paper is not conditionality in practice.

Required position: Conditionality enforced only in service-catchment areas where the referenced facility meets minimum staffing and equipment standards; unconditional transfer with health and education messaging in areas where standards are not met.

Evidence: Indonesia's PKH adopted soft compliance in underserved districts, converting the effective design to a de facto UCT where the supply side could not support the conditionality. This is the design adaptation that the marginal-child hypothesis (Akresh et al., 2013) and the Zomba experiment (Baird et al., 2011) both imply. Conditionality produces human-capital returns where the service exists; where it does not,

conditionality excludes the poorest from the transfer without producing the intended gain. The blended design (conditional where supply permits, unconditional where it does not) preserves the income effect for all beneficiaries and the compliance effect where the compliance can function.

Implementation: The State Cash Transfer Units, in coordination with State Ministries of Health and Education, should conduct a supply-side readiness audit annually. Households in catchment areas that meet the audit standard receive the conditional transfer with compliance monitoring. Households in catchment areas that fail the standard receive the same transfer amount unconditionally, with behavioral messaging (information about the returns to school and health service use) replacing enforcement. The audit creates a dynamic system: as supply improves, areas transition from unconditional to conditional; as supply deteriorates, they transition back.

R3. Implement the blended age-conditional design

Current position: The NASSP co-responsibility (top-up conditional) targets children's school attendance and health visits without age differentiation.

Required position: Hard conditionality for children aged 6 to 14 (primary and lower-secondary school age, where the enrollment effect of conditionality is largest and the adverse effect on teenage pregnancy and marriage is minimal). Soft conditionality with messaging for adolescents aged 15 to 17 (upper-secondary age, where the pregnancy and marriage risk is higher and the enrollment gain from conditionality is smaller). Unconditional transfer for household members aged 18 and above.

Evidence: Baird, McIntosh, and Özler (2011) find that the CCT outperforms the UCT on schooling but the UCT outperforms the CCT on teenage pregnancy and marriage, with the trade-off concentrated among older adolescents. Akresh, de Walque, and Kazianga (2013) find that conditionality is binding for marginal children (girls, younger children, lower-ability children) and inframarginal for non-marginal children. The blended design minimizes the trade-off by applying hard conditionality where it produces the largest enrollment gain at the lowest risk of adverse exclusion, and softening or removing it where the exclusion risk is highest.

R4. Anchor institutional durability in legislation

Current position: NASSCO and NCTO are project-implementation units within the Federal Ministry of Humanitarian Affairs. The January 2024 suspension demonstrated that executive action can interrupt the program without legislative constraint.

Required position: The National Social Protection Act should codify the cash transfer as a statutory entitlement of eligible households, with disbursement continuity protected against executive suspension except by legislative vote. The Act should establish NASSCO as an autonomous agency with a fixed tenure governing board, independent of the ministry of the day, modelled on the South African SASSA governance structure.

Evidence: Brazil's Bolsa Familia survived four administrations because the Cadastro Unico and municipal delivery system created a technically independent infrastructure that could not be dismantled by presidential decree. Mexico's Progresa was renamed twice but not dismantled until AMLO, who replaced it with a larger unconditional program rather than eliminating the transfer. Nicaragua's RPS died when domestic political support evaporated, because it had never built an institutional constituency independent of the executive. The Nigerian record (COPE closed without evaluation, NASSP suspended for 12.5 months) demonstrates the fragility of programs that depend on a single administration's political commitment.

R5. Build evaluation infrastructure from inception

Current position: The NASSP impact evaluation was not completed at project close. The baseline was conducted at midterm. The targeting accuracy failure was documented but not remediated in the scale-up design. No randomized or quasi-experimental evaluation of the Nigerian cash transfer exists.

Required position: A prospective randomized controlled trial or regression discontinuity design embedded in the NASSP-SU from the first disbursement. The RRR's geographic targeting and the PMT cut-off both provide natural identification strategies: the PMT score creates a discontinuity at the eligibility threshold, and the ward-level poverty maps create geographic variation in program exposure. Baseline data collection before first payment. Midline at 12 months. Endline at 36 months. Results published within 12 months of endline.

Evidence: Mexico's Progresa randomized rollout seeded the global evidence base for CCT. Indonesia's PKH clustered RCT produced the strongest health-specific evidence in Asia. Malawi's Zomba experiment produced the definitive conditionality test. Nigeria, with the largest population and the weakest evidence base in the sample, is the country that most needs and can most benefit from rigorous impact evaluation. The PMT cut-off alone provides a regression discontinuity design at zero marginal cost: compare households just above and just below the PMT threshold to estimate the local average treatment effect.

R6. Fix targeting accuracy

Current position: 53 percent of beneficiaries in the bottom two consumption quintiles against an 80 percent target. The NSR coverage is 20 percent of the population but concentrated in rural areas; urban coverage remains below 10 percent.

Required position: Two targeting reforms. First, recalibrate the PMT formula using the 2018/19 NLSS data (the current formula predates this survey). Second, integrate the NSR with the NIN (National Identification Number) database and the CBN financial-inclusion registry to enable cross-registry de-duplication and income verification at the household level.

Evidence: The IEG review documents the targeting failure as the project's most significant design shortfall. The NASSP-SU PAD commits to updating the PMT formula and expanding the RRR for urban targeting, but the commitments have not yet been operationalised. The targeting failure means that nearly half of the program's resources reach households that are not in the bottom two quintiles, diluting the poverty-reduction return per naira spent. The PMT recalibration and the NIN integration are low-cost, high-return reforms: the data already exist, and the integration requires interagency agreement rather than new data collection.

R7. Protect the delivery system from political interference

Current position: The January 2024 suspension froze NASSCO disbursements for 12.5 months. The presidential panel's findings were not published. Over NGN 30 billion was recovered from microcredit streams but not remitted to the Treasury Single Account.

Required position: Three institutional safeguards. First, the GRM (grievance Redress Mechanism) should be fully digitised and publicly accessible, with automatic escalation of unresolved grievances to the legislative oversight committee after 30 days. Second, third-party monitoring reports should be published on a fixed schedule (quarterly) regardless of the implementing ministry's consent. Third, disbursement should be technically automated through the national payments infrastructure (straight-through processing to NUBAN accounts), removing the ministerial discretion to halt payments by withholding approval.

Evidence: The NASSP-SU PAD commits to digitised GRM, third-party monitoring, and straight-through processing. These are design commitments that the suspension experience shows are necessary but not sufficient: the PAD's commitments must be hardened into operational procedures that function independently of the political principal. Brazil's Bolsa Familia achieved this through the Cadastro Unico's municipal-level operation: the federal government transfers funds to municipalities, and the municipality disburses to beneficiaries, a two-step process that neither level can unilaterally halt. Nigeria's federal-to-state-to-LGA structure provides analogous decentralisation, but the current design concentrates disbursement approval at NASSCO/NCTO, creating a single point of political failure.

Table 7: Recommended Architecture

Design element	Current	Recommended	Evidence basis
Transfer size	NGN 5,000/mo (6-7% consumption)	NGN 15,000-20,000/mo (15-20% consumption)	Bastagli et al. 2016; LEAP; CT-OVC
Indexation	None	Semi-annual food CPI, automatic	N-Power erosion; LEAP erosion; PAD annex clause
Conditionality	Soft, weakly enforced	Hard for ages 6-14; soft for 15-17; none for 18+	Baird et al. 2011; Akresh et al. 2013
Supply-side	Not verified before conditionality	Annual readiness audit; blended design	PKH soft compliance; Bolivia Bono failure
Targeting	PMT (53% accuracy, outdated formula)	Recalibrated PMT + NIN integration	IEG 2024 targeting failure
Evaluation	Not completed at project close	Prospective RDD/RCT from first disbursement	Progresa randomized rollout precedent
Institutional anchor	Project unit in ministry	Statutory autonomous agency	Bolsa Familia 4-administration survival
Payment rails	PSP-dependent, ministerial approval	Straight-through to NUBAN, automated	January 2024 suspension; PAD Box 1
GRM	Operational but not digitised	Fully digitised, public, auto-escalation	ICIR 2018; PAD GRM App commitment
Third-party monitoring	Reports exist but not on fixed schedule	Quarterly publication, statutory	NASSP experience; PAD commitment

The recommended architecture does not require a new program. It requires the existing NASSP-SU design to be implemented as its own PAD specifies, with the transfer raised to adequacy, the conditionality adapted to supply-side reality and beneficiary age, the indexation clause operationalised, the PMT recalibrated, and the evaluation conducted from inception rather than deferred. Every element of the recommended architecture has a precedent in the country evidence this brief has reviewed. None requires institutional invention; all require political will and fiscal allocation.

Unimplemented commitments watch list

Four commitments made by the World Bank and by NASSCO between 2016 and 2024 remain unimplemented at the time of writing. Each is documented in the source record and each affects the credibility of subsequent program claims.

- The NASSP impact evaluation, committed in the 2016 PAD (P151488) and reaffirmed under NASSP-SU, has not been completed. Preliminary results were reported at the ICR stage but the final impact evaluation report was not available at project close (F4a, IEG 2024).
- The PMT targeting formula, in use since program launch, predates the 2018/19 Nigeria Living Standards Survey. The NASSP-SU PAD (F4b, 2024) commits to recalibrating the formula using the updated NLSS data. Operationalization has not been reported.
- State-disaggregated beneficiary data are not publicly disclosed by NASSCO. Aggregate national figures appear in project reports; sub-national disaggregation would allow independent verification of the 53 percent targeting accuracy finding at state level. The absence of disclosure is itself a finding.
- The NASSP-SU first disbursement cycle completion status, as of the last publicly available World Bank Implementation Status Report, remains reported as ongoing without completion confirmation.

These four items constitute a shortlist for follow-up reporting. Their resolution or continued non-resolution should be tracked in any subsequent brief in this series.

References

- Akresh, R., de Walque, D. & Kazianga, H. (2013). "Cash Transfers and Child Schooling: Evidence from a Randomized Evaluation of the Role of Conditionality." World Bank Policy Research Working Paper 6340.
- All Progressives Congress (2015). APC Manifesto: My Covenant with Nigerians.
- Baird, S., McIntosh, C. & Özler, B. (2011). "Cash or Condition? Evidence from a Cash Transfer Experiment." *Quarterly Journal of Economics* 126(4): 1709-1753.
- Barber, S. & Gertler, P. (2008). "The Impact of Mexico's Progresa on Birthweight." Unpublished manuscript.
- Bastagli, F., Hagen-Zanker, J., Harman, L., Barca, V., Sturge, G., Schmidt, T. & Pellerano, L. (2016). *Cash Transfers: What Does the Evidence Say?* London: ODI.
- Carvalho, N. & Rokicki, S. (2019). "The Impact of India's Janani Suraksha Yojana Conditional Cash Transfer Program: A Replication Study." *Journal of Development Studies* 55(5): 989-1006.
- de Groot, R. (2016). "Ghana LEAP 1000 Impact Evaluation: Analysis of Transfer Size and Estimated Impacts." UNICEF Innocenti Research Brief 11.
- de Walque, D., Dow, W.H., Nathan, R., Kazunga, R., Baluja, A., Isingo, J., Medard, M., McCormick, T.J., Lazaro, H. & Phillipson, L. (2012). "Incentivizing Safe Sex: A Randomized Trial of Conditional Cash Transfers for HIV and Sexually Transmitted Infection Prevention in Rural Tanzania." *Journal of Development Economics*.
- Fenwick, C. (2009). "Avoiding Governors: The Success of Bolsa Familia." *Journal of Latin American Studies* 41(3): 501-530.
- Fiszbein, A. & Schady, N. with Ferreira, F.H.G., Grosh, M., Keleher, N., Olinto, P. & Skoufias, E. (2009). *Conditional Cash Transfers: Reducing Present and Future Poverty*. Washington, DC: World Bank.
- FIJ (2024). "400,000 N-Power Batch C Stream 2 Beneficiaries Owed N108bn in Arrears." *Foundation for Investigative Journalism*.
- Handa, S., de Hoop, J., Palermo, T. et al. (2016). "Ghana LEAP 1000 Program: Baseline Evaluation Report." UNICEF Office of Research, Innocenti.
- Handa, S., Halpern, C.T., Pettifor, A. et al. (2014). "Impact of the Kenya Cash Transfer for Orphans and Vulnerable Children on Sexual Risk Behaviors." *PLOS ONE* 9(5).
- Hanlon, J., Barrientos, A. & Hulme, D. (2010). *Just Give Money to the Poor: The Development Revolution from the Global South*. Sterling, VA: Kumarian Press.
- Heimo, T. & Syvatera, J. (2023). "The Ghostwriting of a Global Policy Script: International Organizations and the Discursive Construction of Conditional Cash Transfers." *Critical Policy Studies* 16(1): 79-96.
- ICIR (2018). "Investigation: Poorest Nigerians Deprived of Federal Government's Cash Transfer." *International Centre for Investigative Reporting*, July 5.
- IEG (2024). *Implementation Completion and Results Review: Nigeria National Social Safety Net Project (P151488)*. World Bank Independent Evaluation Group.
- IPA & J-PAL (2025). *Social Protection Program Evaluations Catalogue*. Innovations for Poverty Action.
- Kilburn, K., Ferrone, L., Pettifor, A., Wagner, R., Gomez-Olive, F.X. & Kahn, K. (2020). "The Impact of a Conditional Cash Transfer on Multidimensional Deprivation of Young Women: Evidence from South Africa's HPTN 068." *Social Indicators Research* 151: 865-895.

Kusuma, D., Thabrany, H., Hidayat, B., McConnell, M., Berman, P. & Cohen, J. (2017). "New Evidence on the Impact of Large-Scale Conditional Cash Transfers on Child Vaccination Rates: The Case of a Clustered-Randomized Trial in Indonesia." *Journal of Development Economics*.

Levy, S. (2006). *Progress Against Poverty: Sustaining Mexico's Progresa-Oportunidades Program*. Washington, DC: Brookings Institution Press.

Lim, S.S., Dandona, L., Hoisington, J.A. et al. (2010). "India's Janani Suraksha Yojana, a Conditional Cash Transfer Program to Increase Births in Health Facilities: An Evaluation of the Effect on Perinatal Mortality and Neonatal Morbidity." *The Lancet* 375(9730): 200-207.

Lindert, K., Linder, A., Hobbs, J. & de la Briere, B. (2007). "The Nuts and Bolts of Brazil's Bolsa Familia Program: Implementing Conditional Cash Transfers in a Decentralized Context." *World Bank SP Discussion Paper* 0709.

Mashood, U.A. (2026). *An Independent M&E Review of Nigeria's N-Power Program, 2016-2026: Origin, Performance, and Reform Options*. Social Protection M&E Nigeria series, Brief 4.

NASSCO (2025). *Statement on Digital Payment Transition and NIN Verification*. National Social Safety Nets Coordinating Office.

Ngamasana, F. & Moxon, S. (2024). "Cash Transfer, Maternal and Child Health Outcomes: A Scoping Review in Sub-Saharan Africa." *Global Health Action* 17(1).

Nigerian Tribune (2025). "Reps Probe Over N30bn Seized National Social Investment Program Fund."

Office of the SGF (2024). *Statement on Suspension of NSIP Programs*. Secretary to the Government of the Federation.

Okoli, U., Morris, L., Oshin, A., Pate, M.A., Aigbe, C. & Muhammad, A. (2014). "Conditional Cash Transfer Schemes in Nigeria: Potential Gains for Maternal and Child Health Service Uptake in a National Pilot Program." *BMC Pregnancy and Childbirth* 14: 39.

Osimen, G.U., Agbonkhese, O. & Adesina, F. (2025). "Effect of N-Power Program on Youth Empowerment in Lagos State, Nigeria." *Journal of Public Administration*.

Parker, S.W. & Vogl, T. (2018). "Do Conditional Cash Transfers Improve Economic Outcomes in the Next Generation? Evidence from Mexico." *NBER Working Paper* 24303.

Sahara Reporters (2022). "N-Power: D'banj Detained by ICPC Over Alleged Diversion of N-Power Funds."

Stampini, M. & Tornarolli, L. (2012). "Labor Market Participation, Poverty, and Inequality in Latin America: The Case of Conditional Cash Transfer Programs." *IADB Working Paper*.

TheCable (2019, 2021). *Undercover Investigation into N-Power Redeployment Bribe Economy and Ghost Beneficiaries*.

Villatoro, P. (2005). "Conditional Cash Transfer Programs: Experiences from Latin America." *CEPAL Review* 86: 83-103.

World Bank (2014). *Overview of the Philippines' Conditional Cash Transfer Program: The Pantawid Pamilyang Pilipino Program*. Washington, DC.

World Bank (2016). *Project Appraisal Document on a Proposed Credit to the Federal Republic of Nigeria for the National Social Safety Nets Project (P151488)*. Washington, DC.

World Bank (2024). *Project Appraisal Document on a Proposed Credit for the National Social Safety Net Program Scale Up (P176935)*. Washington, DC.

Annexes

Annex A. Full Source Matrix (26 Sources, Condensed)

Foundational Sources (F)

ID	Citation	Type	Sections served	Key contribution
F1	Fiszbein, A. & Schady, N. (2009). Conditional Cash Transfers: Reducing Present and Future Poverty. Washington, DC: World Bank.	World Bank Policy Research Report	1, 2, 3, 9	300-page foundational review; design rationale; institutional shift chapter
F2	Bastagli, F. et al. (2016). Cash Transfers: What Does the Evidence Say? London: ODI.	Rigorous evidence reviews (201 studies)	5, 6, 7, 9, 10A, 10	Pooled evidence across 6 outcome domains; threshold analysis
F3	Levy, S. (2006). Progress Against Poverty: Sustaining Mexico's Progres-Oportunidades Program. Washington, DC: Brookings.	Book	3	Architect's own account of Progres design
F4a	IEG (2024). Implementation Completion and Results Review: National Social Safety Net Project. World Bank.	Independent evaluation	8, 9	NASSP ICR: SATISFACTORY outcome, targeting 53%, co-responsibility 6%
F4b	World Bank (2024). National Social Safety Net Program Scale-Up (P176935), Project Appraisal Document.	PAD	8, 11	USD 800M; NGN 5,000/month; 3 components; risk SUBSTANTIAL
F5	Fenwick, C. (2009). "Avoiding Governors: The Success of Bolsa Familia." Journal of Latin American Studies 41(3).	Peer-reviewed	1, 4	Brasilia and Campinas pre-history; central-local mechanics
F6	Hanlon, J., Barrientos, A. & Hulme, D. (2010). Just Give Money to the Poor. Kumarian Press.	Book (excerpt)	1, 2	Global South led narrative; paradigm-level statistics

Country Story Sources (S)

ID	Citation	Country	Program	Sections
S1	Parker, S.W. & Vogl, T. (2018). NBER WP 24303.	Mexico	Progres	3, 9
S2	Lindert, K. et al. (2007). World Bank SP DP 0709.	Brazil	Bolsa Familia	4, 9
S3	Villatoro, P. (2005). CEPAL Review 86.	Regional	Multiple	5
S4	World Bank (2014). Overview of Pantawid Pamilya.	Philippines	4Ps	6, 9
S5	Kusuma, D. et al. (2017). J. Dev. Econ.	Indonesia	PKH	6, 9
S6	Carvalho, N. & Rokicki, S. (2019). J. Dev. Studies 55(5).	India	JSY	6, 9
S7	Handa, S. et al. (2016). UNICEF Innocenti.	Ghana	LEAP	7, 9
S8	Handa, S. et al. (2014). PLOS ONE.	Kenya	CT-OVC	7, 9
S9	Baird, S., McIntosh, C. & Özler, B. (2011). QJE 126(4).	Malawi	Zomba RCT	7, 9
S10	Kilburn, K. et al. (2020). Social Indicators Research 151.	South Africa	HPTN 068 + CSG	7, 9
S11	de Walque, D. et al. (2012).	Tanzania	PSSN	7, 9

Nigeria Sources (N)

ID	Citation	Type	Key content
N1	Okoli, U. et al. (2014). BMC Pregnancy Childbirth 14:39.	Peer-reviewed	COPE 2007; 22K HH; SURE-P pilot; closed without evaluation
N2	ICIR (2018). Investigation: Poorest Nigerians Deprived of FG's Cash Transfer.	Investigative journalism	Payment delays; exclusion errors; no grievance mechanism
N3	Mashood, U.A. (2026). Brief 4: An Independent M&E Review of N-Power.	Sibling brief	Stipend erosion 82-90%; ghost beneficiaries 9-18%; suspension 12.5 months

Cross-Cutting Sources (X)

ID	Citation	Type	Key content
X1	Bastagli et al. (2016) Ch 7 + #21.	Systematic review	CCT vs UCT on schooling; 60% enrollment odds improvement with enforcement
X2	#127 + Ngamasana & Moxie (2024). Global Health Action 17(1).	Systematic + scoping review	SSA: CTs increase health utilization; conditionality weak; supply-side binding

Supporting Sources

ID	Citation	Key content
#53	Akresh, de Walque & Kazianga (2013). WPS6340.	Burkina Faso 4-arm RCT; CCT more effective for marginal children
#135	Heimo & Syvatera (2023). Critical Policy Studies 16(1).	Ghostwriting of CCT narrative by IOs
Barber & Gertler (2008)	Birthweight impact of Progres.	+127.3g birthweight, 4.6pp reduction in low birthweight
Lim et al. (2010)	JSY Lancet study.	Institutional delivery increased; replicated by Carvalho & Rokicki (2019)
Stampini & Tornarolli (2012)	Regional aggregate.	15-20M HH across Latin America by 2010

Annex B. CCT Program Classification by Type and Outcome

Program	Country	Year	Primary type	Enrollment effect	Health utilization effect	Nutrition effect	Economic effect	Nigeria has indicator?
Progresa	Mexico	1997	Education + Health	Strong (+12%)	Strong (+18% visits)	Moderate (↓ stunting)	Weak (long-term for women)	Yes (NSR enrollment data)
Bolsa Familia	Brazil	2003	Education + Health (soft)	Moderate	Moderate	Moderate	Moderate (21% Gini)	Yes (NSR)
Familias en Accion	Colombia	2000	Education + Health	Strong (+7-10%)	Moderate	n/a	Weak	Partial
4Ps	Philippines	2007	Education + Health + Nutrition	Moderate	Moderate	Moderate	Weak	Yes (NSR)
PKH	Indonesia	2007	Health + Education	Moderate	Strong (↑ vaccination)	n/a	Weak	Partial
JSY	India	2005	Health (single condition)	n/a	Strong (↑ facility delivery)	n/a	Weak	Yes (MCH data)
LEAP	Ghana	2008	Education + Health (soft)	Moderate	Moderate (↑ ANC)	None (stunting)	Moderate (↓ poverty)	Partial
CT-OVC	Kenya	2007	Unconditional	Moderate	Moderate	None (stunting)	Moderate (↓ sexual debut)	Partial
Zomba CCT	Malawi	2007	Education (RCT)	Strong (+43% vs UCT)	n/a	n/a	n/a	No
HPTN 068	South Africa	2011	Education (RCT)	n/a	n/a	n/a	Moderate (↓ deprivation)	No
PSSN	Tanzania	2013	Hybrid (PW + CT)	Weak	Transient (HIV)	n/a	Weak (PW quality)	Partial
NASSP-HUP	Nigeria	2016	Education + Health (soft)	***Not measured**	***Not measured**	***Not measured**	***Not measured**	***No evaluation completed**

Key insight: Every program that measured its education or health effect found a positive result. Nigeria is the only program in the sample that has not measured any outcome. The absence of evidence is the central finding, not the absence of effect.

Annex C. Glossary of Acronyms and Terms

Acronym / Term	Definition
CCT	Conditional Cash Transfer. Cash payment to poor households conditional on specified behaviors (typically school attendance and health visits).
UCT	Unconditional Cash Transfer. Cash payment to poor households with no behavioral conditions.
PMT	Proxy Means Test. Targeting method using observable household characteristics to predict income/consumption.
NSR	National Social Register. Nigeria's registry of poor and vulnerable households.
NASSCO	National Social Safety Nets Coordinating Office. Federal coordinating agency for Nigeria's safety net programs.
NCTO	National Cash Transfer Office. Federal implementing agency for Nigeria's cash transfer program.
NASSP	National Social Safety Net Project. World Bank IDA-funded project (2016-2022).
NASSP-SU	National Social Safety Net Program Scale-Up. Follow-on project (2022-present), USD 800M IDA.
ESR-CT	Economic Shock Responsive Cash Transfer. NASSP-SU Component 1: temporary 6-month transfer.
ER-CT	Extended Regular Cash Transfer. NASSP-SU Component 2: extension for existing beneficiaries.
RRR	Rapid Response Register. Urban targeting mechanism using satellite imagery and USSD self-registration.
GRM	Grievance Redress Mechanism. Complaint and resolution system for beneficiaries.
IEG	Independent Evaluation Group. World Bank's independent evaluation arm.
PAD	Project Appraisal Document. World Bank project design document.
RDD	Regression Discontinuity Design. Quasi-experimental identification strategy using program eligibility cut-offs.
RCT	Randomized Controlled Trial. Gold-standard evaluation design with random assignment.
COPE	In Care of the People. Nigeria's first CCT pilot (2007-2015).
SURE-P	Subsidy Re-Investment and Empowerment Program. Jonathan administration's post-fuel-subsidy program.
NSIPA	National Social Investment Programs Agency. Statutory agency (2023 Act) for Nigeria's social investment portfolio.
TSA	Treasury Single Account. Nigeria's consolidated government revenue account.
NIN	National Identification Number. Unique biometric identity number issued by NIMC.
FGT	Foster-Greer-Thorbecke. Class of poverty measures (headcount, poverty gap, squared gap).
HH	Household.
ANC	Antenatal Care.
MCH	Maternal and Child Health.
PHC	Primary Health Centre.
LGA	Local Government Area. Nigeria's third tier of government (774 total).
NLSS	Nigeria Living Standards Survey. National household survey (2018/19 most recent).